



Covering Letter

Date:-

To,
Medi Assist Insurance TPAPvt.LtdCa
shlessProcessingCentre
#58/1A, Singasandra, Hosur Main Road, Be
gur Post, Bangalore. PIN-560068.
Helpline: 080-22068666

This is to bring to your kind attention that we have enclosed a
claim document/sof your member who have availed cashless treatment based on your approval.

Details are as below.

Claim No.	Patient Name	Bill No.	IP No.	Date of Admission	Date of Discharge	Total Bill Amount	Total Approved Amount	Total Patient Paid Amount	Total Claiming Amount
13216 2419	Atoke Kc Mukherjee	261 8927029	25/14	27.6.25	3.7.25	139165	111058		

There is a Base/Top-up claim associated with this claim, and the claim number is _____

Documents Check List	tick (✓) to confirm attachment
Final PA Letter	
PA request form	✓
Consultation letter and investigations before admission	
CKYC form with photo of the primary policyholder	✓
PAN Card copy of Primary policyholder	
Aadhar Card copy of primary policyholder	
Patient ID proofs (Aadhar and PAN Card)	✓
Final bill & breakup	
Discharge summary	
Investigations/lab report	✓
Implant stickers	
Implant invoice	✓

Thanking you,

Sender Details:-

Name:-

Designation:-

Phone No:-

Email:-

Shilpi Adhikary
TPA Coordinator



A Unit of Parashmani Medical Centre Pvt. Ltd.

Hospital Address: C-49, Commercial Area, Opp. ESIC Regional Office, City Centre, Durgapur-713216, W.B., India • Regd. Office: 139D, Rashbehari Avenue, Ground Floor, Kolkata - 700 029

Website: www.healthworldhospitals.com, Email: info@hwhos.com, CIN: U85100WB2014PTC201736 PAN: AAHCP6873G

For any query, Pl. Call: 8170055555 / 8170066660 / 0343-2547755, For Ambulance, Call: 8170052660

For Blood Collection from Home, Contact: 8170052815 (Durgapur), 8170052700 (Asansol), 8170052680 (Dhanbadi)



Medi Assist Insurance TPA Pvt. Ltd

TPA Logo Medi Assist Logo Plan Hospitalization Hospital



XAP132162419

Date :03 Jul 2025 11:46

To,

The Administrator / Medical Superintendent,
Healworld Hospitals (A Unit Of Parashmani Medical Centre Pvt Ltd),
Plot no. C-49, Commercial Area, Opp to ESIC Regional Office, City Centre,
Hospital ID: (216558)
Rohini Id: 8900060380189

Dear Partner,

With reference to your request (132162419) for final cashless pre-authorization, we hereby authorize INR **111058** against your final bill amount INR **139165**.
The details of the pre-authorization are as follows:

Patient Details

Patient Name	Aloke Kumar Mukherjee
Relation to Primary Beneficiary	Father-in-law
Age	65
Gender	M
Insurance Company	The New India Assurance Co. Ltd
Medi Assist ID	4021570814
Policy Holder	Tata Consultancy Services Ltd
IP No	
Policy No.	92000063250400000025
Policy/Plan Period	01 Apr 2025 to 31 Mar 2026
Primary Beneficiary	Sourav Mahapatra
Insurer Claim No	TP00392000025901027843
Insurer Member ID	

Treatment Details

Provisional Diagnosis	Liver disease, unspecified
Expected/Actual Date Of Admission	27 Jun 2025
Treating Doctor	T N CHATTAPADHYAY
Procedure / Treatment Planned	Conservative Management
Estimated/Actual Date of Discharge	03 Jul 2025
Room Category Occupied	General/Economy ward
Length Of Stay	6
Eligible Room Category	Sharing Ward (Semi-private / Multibed Ward)

Total Authorized amount Rs 111058 (One Lakh Eleven Thousand and Fifty Eight).

Authorization Remarks :

Approved as per policy T&C. Rs 10000 deducted as 10% or maximum up to Rs. 10000 copay applicable as per policy T & C.

Note: If Top Up is available and applicable, as per policy conditions, Top Up claims will be processed and additional amounts will be approved along with base amount as per your benefit.

Authorization Summary

Total bill amount (INR)	139165
Other Deductions(INR)*	8285
Copay (INR)	10000
Hospital Discount (INR)	9877
Deductibles (INR)	0
Total Authorized Amount(INR)	111058

Detailed list of deductions have been shared with the claimant

Terms and conditions for authorization:

1. Cashless authorization letter issued on the basis of information provided in pre authorization form. In case of misrepresentation/concealment of facts, any material difference/deviation/ discrepancy in information is observed in discharge summary / IPD records then cashless authorization stands null & void. At any point of claim processing Insurer or TPA reserves the right to raise queries for any other document to ascertain the admissibility of claim.
2. KYC (know your customer) details of proposer/employee/beneficiary are mandatory for claim payout above Rs. 1 lakh.
3. Network provider shall not collect any additional amount from the individual in excess of Agreed Package Rates except cost towards non admissible amounts (including additional charges due to opting higher room rent than eligibility/choosing separate line of treatment which is not envisaged/considered in Package)
4. Network provider shall not make any recovery from the deposit amount collected from the insured except for the cost towards non admissible amounts (including additional charges due to opting higher room rent than eligibility/choosing separate line of treatment which is not envisaged/considered in Package)
5. In the event of unauthorized recovery of any additional amount from the insured in excess of Agreed Package Rates, the authorized TPA/Insurance company reserves the right to recover the same or get the same refunded to the policy holder from the network provider and/or take necessary action as provided under the MOU.
6. Where treatment / procedure to be carried out by a Doctor/Surgeon of insured's choice (not empaneled with the Hospital) network provider may give treatment after obtaining specific consent of the policyholder.
7. Expenses on investigations / diagnostic tests, etc. which are not related to the condition for which admission is sought are not admissible
8. Expenses are excluded which are not covered / not payable as per health insurance policy terms and conditions are not admissible
9. Expenses related to medicines/drugs incurred post discharge and Differential cost borne by the policyholder may be reimbursed by Insurer subject to terms and conditions of the policy

The following documents must be submitted in full within 7 days from date of discharge to enable settlement of claim:

1. Original cashless claim form in IRDAI format
2. Government ID proof and Medi Assist ID card of the patient along with KYC form
3. Detailed discharge summary with Main hospital bill along with Break-up of the bill amount being claimed
4. Cash memos from the Hospitals / Chemists supported by proper prescriptions
5. Diagnostic Test Reports, X-ray films, and Receipts supported by note from the attending Medical Practitioner / Surgeon recommending such diagnostic tests
6. Original sticker for all the implants & high value consumables
7. Surgeon's Certificate stating the nature of operation performed and Surgeon's Bill and Receipt
8. Certificates from attending Medical Practitioner / Surgeon giving patient's condition and advice on discharge.
9. Copy of the receipt for the amount settled by the patient / representative
10. Final hospital bills should be issued in the name of **The New India Assurance Co. Ltd** as a payer for payment of cashless claims. This is a mandatory requirement for claim settlement.
11. Please send cashless documents to the address mentioned in the last page of the letter. (Beneath signature)

Note: As per Modified Guidelines on Standards and Benchmarks for Hospitals in the Provider Network issued by IRDAI vide Circular Ref: IRDAI/HLT/REG/GDL/114/07/2018 dated 27th July 2018, your Hospital is mandatorily required to Register with ROHINI and obtain either Pre-entry level Certificate (or higher level of certificate) issued by NABH or State Level Certificate (or higher level of certificate) under NQAS, issued by National Health Systems Resources Centre (NHSRC) on or before July 26, 2019.

QUICK LINKS:

For partner hospital

View this claim on [IHX](#). Not on IHX yet? [Sign Up](#) now.

Warm Regards,



Medi Assist Insurance TPA Pvt. Ltd
 CIN: U85199KA1999PTC025678
 Cashless Processing Centre
 #58/1A, Singhasantra
 Hesar Man Road,
 Bengaluru Post
 Bengaluru PIN - 560068
 Helpline: 0120-4927324

Disclaimer: The TPA extends the cashless facility subject to the standard terms & conditions of the policy and the information provided in the cashless request form. We suggest that the patient continues with the treatment as advised by the treating doctor, irrespective of the pre-authorization/cashless facility.

App 

Connect 

THIS IS A SYSTEM GENERATED CORRESPONDENCE. PLEASE DO NOT REPLY TO THIS EMAIL

APP

ICU10-5

390518



REQUEST FOR CASHLESS HOSPITALISATION FOR HEALTH INSURANCE POLICY PART C (Revised)

TO BE FILLED IN BLOCK LETTERS

Name of the hospital: **HEALTHWORLD HOSPITALS**
 Hospital location: **DURGAPUR** Hospital ID: **10000000000000000000**
 Hospital email ID: **insurance@hwhos.com** ROHINI ID: **8900080380189**

DETAILS OF THIRD PARTY ADMINISTRATOR

a) Name of TPA company: **Medi Assist Insurance TPA Pvt Ltd** b) Phone no: **080 22068666** c) Toll Free Fax no: **1800 425 9559**

TO BE FILLED BY INSURED/PATIENT

✓ a) Name of the patient: **ALOK KUMAR MUKHERJEE**
 ✓ b) Gender: ☒ Male ☐ Female ☐ Third gender ✓ c) Contact no: **9775509321** ✓ d) Alternate contact no: **8583042932**
 ✓ e) Age: Years **25** Months **09** f) Date of birth: **05071960** ✓ g) Insurer ID card no: **10000000000000000000**
 h) Policy number/Name of corporate: **10000000000000000000** i) Employee ID: **10000000000000000000**
 j) Currently do you have any other medical claim/health insurance: ☐ Yes ☐ No j.1) Insurer name: **10000000000000000000**
 (2) Give details: **10000000000000000000**
 k) Do you have a family physician, if yes: Name: **10000000000000000000** k.1) Contact no: **10000000000000000000**
 l) Occupation of insured patient: **10000000000000000000**
 m) Address of insured patient: **10000000000000000000**

TO BE FILLED BY THE TREATING DOCTOR/HOSPITAL

a) Name of the treating doctor: **DR. P. R. Nayak** b) Contact no: **10000000000000000000**
 c) Name of illness/disease with presenting complaints: **Complaint of Sudden onset of heaviness discomfort & SOB** d) Relevant clinical findings: **10000000000000000000**
 e) Duration of the present ailment: **10** days e.1) Date of first consultation: **DDMMYYYY**
 e.2) Past history of present ailment if any: **10000000000000000000**
 f) Provisional diagnosis: **Junctional Bradycardia / Dysrhythmia** f.1) ICD 10 code: **10000000000000000000**
 g) Proposed line of treatment: ☒ Medical management ☐ Surgical management ☒ Intensive care ☐ Investigation ☐ Non-Allopathic treatment
 h) If investigation and/or medical management, provide details: **10000000000000000000** h.1) Route of drug administration: ☐ IV ☐ Oral ☐ Other **10000000000000000000**
 i) If Surgical, name of surgery: **10000000000000000000** i.1) ICD 10 PCS code: **10000000000000000000**
 j) If other treatments provide details: **10000000000000000000** k) How did injury occur: **10000000000000000000**
 l) In case of accident: i. Is it RTA: ☐ Yes ☐ No ii. Date of injury: **DDMMYYYY** iii. Reported to Police: ☐ Yes ☐ No iv. FIR no: **10000000000000000000**
 v. Injury/Disease caused due to substance abuse/alcohol consumption: ☐ Yes ☐ No vi. Test conducted to establish this, if yes attach reports: ☐ Yes ☐ No
 m) In case of maternity: G **10** P **10** L **10** A **10** n) Expected date of delivery: **DDMMYYYY**

DETAILS OF THE PATIENT ADMITTED

✓ a) Date of admission: **27082025** b) Time of admission: **HHMM** c) This is ☒ an emergency/ ☐ a planned hospitalization event
 d) Expected no. of days stay in hospital: **4-5** Days e) Days in ICU: **10** Days f) Room type: **IU**



REQUEST FOR CASHLESS HOSPITALISATION FOR HEALTH INSURANCE POLICY

PART C (Revised)

TO BE FILLED IN BLOCK LETTERS

g) Per Day Room Rent + Nursing & Service charges + Patient's Diet:

Rs. 40000/-

h) Expected cost for investigation + diagnostics:

Rs. 17000/-

i) ICU Charges:

Rs. 0000/-

j) OT Charges:

Rs. 0000/-

k) Professional fees Surgeon + Anesthetist fees + Consultation charges:

Rs. 6000/-

l) Medicines + Consumables cost of Implants: (specify if applicable)

Rs. 29000/-

m) Other hospital expenses if any:

Rs. 0000/-

n) All inclusive package charges if any applicable:

Rs. 0000/-

o) Sum Total expected cost of hospitalization

Rs. 93000/-

p. Mandatory past history of any chronic illness. If yes (since month/year)

☐ 1. Diabetes	M	M	Y	Y
☐ 2. Heart Disease	M	M	Y	Y
☐ 3. Hypertension	M	M	Y	Y
☐ 4. Hyperlipidemias	M	M	Y	Y
☐ 5. Osteoarthritis	M	M	Y	Y
☐ 6. Asthma/ COPD / Bronchitis	M	M	Y	Y
☐ 7. Cancer	M	M	Y	Y
☐ 8. Alcohol or drug abuse	M	M	Y	Y
☐ 9. Any HIV or STD / related ailments	M	M	Y	Y
☐ 10. Any other ailment give details:				

DECLARATION (PLEASE READ VERY CAREFULLY)

We confirm having read understood and agreed to the declaration of this form

a) Name of the treating doctor: DR. P. R. NAYAK
b) Qualification:
c) Registration No. with State code:

DECLARATION BY THE PATIENT / REPRESENTATIVE

- I agree to allow the hospital to submit all original documents pertaining to hospitalization to the insurer/TPA after the discharge. I agree to sign on the Final Bill & the Discharge Summary, before my discharge.
- Payment to hospital is governed by the terms and conditions of the policy. In case the insurer / TPA is not liable to settle the hospital bill, I undertake to settle the bill as per the terms and conditions of the policy.
- All non-medical expenses and expenses not relevant to current hospitalization and the amounts over & above the limit authorized by the insurer/TPA not governed by the terms and conditions of the policy will be paid by me.
- I hereby declare to abide by the terms and conditions of the policy and if at any time the facts disclosed by me are found to be false or incorrect I forfeit my claim and agree to indemnify the insurer / TPA.
- I agree and understand that TPA is in no way warranting the service of the hospital & that the insurer / TPA is in no way guaranteeing that the services provided by the hospital will be of a particular quality or standard.
- I hereby warrant the truth of the foregoing particulars in every respect and I agree that if I have made or shall make any false or untrue statement, suppression or concealment with respect to the claim, my right to claim reimbursement of the said expenses shall be absolutely forfeited.
- I agree to indemnify the hospital against all expenses incurred on my behalf, which are not reimbursed by the insurer/ TPA.
- I/We authorize Insurance Company/TPA to contact me/us through mobile/email for any update on this claim.

✓ Patient's / Insured's name: ALOK KUMAR NUKHERJEE
✓ Contact number: 9725509321
✓ Patient's / Insured's signature: Alok Kumar Mukherjee
c) Email ID: (Optional)
Date: DDMMYYYY Time: HHMM

HOSPITAL DECLARATION

- We have no objection to any authorized TPA / Insurance Company official verifying documents pertaining to hospitalization.
- All valid original documents duly countersigned by the insured / patient as per the checklist below will be sent to TPA/ Insurance Company within 7 days of the patient's discharge.
- We agree that TPA / Insurance Company will not be Liable to make the payment in the event of any discrepancy between the facts in this form and discharge summary or other documents.
- The patient declaration has been signed by the patient or by his representative in our presence.
- We agree to provide clarifications for the queries raised regarding this hospitalization and we take the sole responsibility for any delay in offering clarifications.
- We will abide by the terms and conditions agreed in the MOU.
- We confirm that no additional amount would be collected from the insured in excess of Agreed Package Rates except costs towards non-admissible amounts (including additional charges due to opting higher room rent than eligibility choosing separate line of treatment which is not envisaged/ considered in package).
- We confirm that no recoveries would be made from the deposit amount collected from the insured except for costs towards non-admissible amounts (including additional charges due to opting higher room rent than eligibility/ choosing separate line of treatment which is not envisaged/considered in package).
- In the event of unauthorized recovery of any additional amount from the insured in excess of Agreed Package Rates, the authorized TPA / Insurance Company reserves the right to recover the same from us (the Network Provider) and/or take necessary action, as provided under the MOU or applicable laws.

DOCUMENTS TO BE PROVIDED BY THE HOSPITAL IN SUPPORT OF THE CLAIM

- Detailed Discharge Summary and all Bills from the hospital.
- Cash Memos from the Hospitals / Chemists supported by proper prescription.
- Receipts and Pathological Test Reports from Pathologists. Supported by note from the attending Medical Practitioner / Surgeon recommending such pathological Tests.
- Surgeon's Certificate stating nature of Operation performed and Surgeon's Bill and Receipt.
- Certificates from attending Medical Practitioner / Surgeon that the patient is fully cured.

Hospital seal:

Doctor's signature:

Date: DDMMYYYY Time: HHMM

CENTRAL KYC REGISTRY | Know Your Customer (KYC) Application Form | Individual

Important Instructions:

- A) Fields marked with * are mandatory fields.
 B) Tick '✓' wherever applicable.
 C) Please fill the form in English and in BLOCK letters.
 D) Please fill the date in DD-MM-YYYY format.
 E) For particular section update, please tick (✓) in the box section number and strike off the sections not required to be updated.

- F) Please read section wise detailed guidelines / instructions at the end.
 G) List of State / U.T code as per Indian Motor Vehicle Act, 1988 is available at the end.
 H) List of two character ISO 3166 country codes is available at the end.
 I) KYC number of applicant is mandatory for update application.
 J) The 'OTP based E-KYC' check box is to be checked for accounts opened using OTP based E-KYC in non-face to face mode



For office use only

Application Type*

- ☐ New ☐ Update

(To be filled by financial institution)

KYC Number

(Mandatory for KYC update request)

Account Type*

- ☐ Normal ☐ Minor ☐ Aadhaar OTP based E-KYC (in non-face to face mode)

1. PERSONAL DETAILS* (Please refer instruction A at the end)

	Prefix	First Name	Middle Name	Last Name
✓ Name* (Same as ID proof)		SOURAV		MAHAPATRA
Maiden Name				
✓ Father / Spouse Name		SAMIRAN		MAHAPATRA
✓ Mother Name		PRATIMA		MAHAPATRA
✓ Date of Birth*		28-09-1987		
✓ Gender*		☒ M-Male ☐ F-Female ☐ T-Transgender		
✓ PAN*		ABYPM0211H		☐ Form 60 furnished

2. PROOF OF IDENTITY AND ADDRESS* (Please refer instruction B at the end)

I. Certified copy of OVD or equivalent e-document of OVD or OVD obtained through digital KYC process needs to be submitted (anyone of the following OVDs)

- ☐ A- Passport Number
☐ B-Voter ID Card
☐ C-Driving Licence
☐ D-NREGA Job Card
☐ E-National Population Register Letter
☒ F-Proof of Possession of Aadhaar
☐ E-KYC Authentication
☐ Offline verification of Aadhaar



Address

Line 1*	15/2 VIVELANAKA ROAD		
Line 2	DURGAPUR-9		
Line 3			
District*	BURDWAN	Pin/Post Code*	713204
		State/U.T Code*	
		City / Town / Village*	
		ISO 3166 Country Code*	

3. CURRENT ADDRESS DETAILS (Please refer instruction B at the end)

☐ Same as above mentioned address (In such cases address details as below need not be provided)

I. Certified copy of OVD or equivalent e-document of OVD or OVD obtained through digital KYC process needs to be submitted (anyone of the following OVDs)

- ☐ A- Passport Number
☐ B-Voter ID Card
☐ C-Driving Licence
☐ D-NREGA Job Card
☐ E- National Population Register Letter
☐ F- Proof of Possession of Aadhaar
☐ E-KYC Authentication
☐ Offline verification of Aadhaar
☐ Deemed Proof of Address - Document Type code
☐ Self Declaration

Line 1*			
Line 2			
Line 3			
District*		Pin / Post Code*	
		State/U.T Code*	
		City / Town / Village*	
		ISO 3166 Country Code*	

☐ 4. CONTACT DETAILS (All communications will be sent to Mobile number/ Email-ID provided) (Please refer instruction C at the end)

Tel. (Off) - Tel. (Res) - Mobbão -
Email ID

☐ 5. REMARKS (if any)

6. APPLICANT DECLARATION

- I hereby declare that the details furnished above are true and correct to the best of my knowledge and belief and I undertake to inform you of any changes therein, immediately. In case any of the above information is found to be false or untrue or misleading or misrepresenting, I am aware that I may be held liable for it.
- I hereby consent to receiving information from Central KYC Registry through SMS/Email on the above registered number/email address.

Date:

DD	MM	YY	YY
----	----	----	----

 Place:

--	--	--	--	--	--	--	--

✓ Sourav Mahapatra

Signature / Thumb Impression of Applicant

7. ATTESTATION / FOR OFFICE USE ONLY

Documents Received ☐ Certified Copies ☐ E-KYC data received from UIDAI ☐ Data received from Offline verification ☐ Digital KYC Process
☐ Equivalent e-document ☐ Video Based KYC

KYC VERIFICATION CARRIED OUT BY

Date		-		-		
Emp. Name						
Emp. Code						
Emp. Designation						
Emp. Branch						

INSTITUTION DETAILS

[illegible]



PPN NETWORK-DECLARATION BY PATIENT/PATIENT'S ATTENDANT

Name of the Hospital:..... Date :

Address:.....

PATIENT NAME (BLOCK LETTERS):..... AGE/SEX :

IP No : UHID No : Mobile No of Patient :

Date of Admission : Time of Admission :

Date of Discharge:..... Time of Discharge:.....

Address of the Patient :

NAME OF THE ATTENDANT : Relationship with the Patient:.....

Mobile No. of Attendant : Address:.....

Declaration regarding Insurance Policy (Strike off the option which is not applicable)

- (i) **Declaration when patient has no insurance policy:**
- I declare that I do not have any insurance policy.

- (ii) **Declaration when patient has insurance policy:**
- I declare that I have following Insurance Policies

Policy No/TPA card No:

Insurance Company:

2) Whether patient opted for Eligible Room Category under
Policy: Yes / No

3) In case, policyholder wishes to avail better facility:

Name of the Additional Facility/ Provision/ Procedure/ Treatment

..... which costs
Rs : (In words:

.....
.....
.....) only.

On my own option, I wish to avail above better facility and I hereby agree to pay on my free will, after being explained in detail by the Hospital authority in my own and understandable language about the above mentioned Additional Facility/Procedure/Treatment and associated cost of it, which is over and above the agreed PPN tariff. Further, if I opt to go for final bill reimbursement with insurance company, respective insurance company will reimburse only as per agreed PPN tariff rates and balance amount will be borne by myself or patient only.

I have also been explained that when room service of a category better than eligible room rent is availed by the patient, not only the difference in room rent but also an equal proportion of all other charges associated with the treatment shall be borne by me.

Signature :

Name of the Patient/Patient's attendant:

Signature :

Name of the Hospital Representative & Hospital Seal :



ভারত সরকার

Indian Identification Authority of India
Government of India

অধিদপ্তর নং: ১০/২০০৮২৮১৮১

To
The
SECY/ANUPATNA
152 VINDHANAKA ROAD
DURGAPUR-4
Durgapur One Town Ward
Burdwan Roadshow
West Bengal 713204
9300060901
MR.03300060901



আপনার আধার সংখ্যা / Your Aadhaar No. :

5018 7725 1493

আধার - সামগ্রিক মানবের অধিকার



SECY/ANUPATNA
The / Office Secy
FBIHQ / ANUPATNA
SECY/ANUPATNA
SECY/ANUPATNA



5018 7725 1493

আধার - সামগ্রিক মানবের অধিকার



তথ্য

- আধার খাতিরের প্রমাণ, নাগরিকত্বের প্রমাণ।
- খাতিরের প্রমাণ অস্বীকার্য, প্রমাণিকরণ করা হয়।

INFORMATION

- Aadhaar is proof of Identity, not of citizenship.
- To establish identity, authenticable online.

- আধার হলো (কিন্তু নয়) নাগরিকত্বের প্রমাণ।
- আধার খাতিরের প্রমাণ অস্বীকার্য ও প্রমাণিকরণ করা হয়।
- আধার সারা দেশেই প্রযোজ্য।
- Aadhaar will be helpful in availing Government and Non-Government services in future.



SECY/ANUPATNA
The / Office Secy
FBIHQ / ANUPATNA
SECY/ANUPATNA
SECY/ANUPATNA

5018 7725 1493

আধার - সামগ্রিক মানবের অধিকার

आयकर विभाग

INCOME TAX DEPARTMENT



भारत सरकार

GOVT. OF INDIA

SOURAV MAHAPATRA

SAMIRAN MAHAPATRA

28/04/1987

Permanent Account Number

AQYPM0211H

Sourav Mahapatra

Signature



09102007



भारत सरकार
Government of India

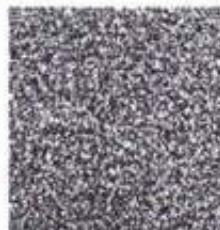
भारतीय विशिष्ट पहचान प्राधिकरण
Unique Identification Authority of India

Enrolment No.: 0000/00615/61738

Download Date: 30/12/2019

To
Aloke Kumar Mukherjee
C/O S/O GOSAIN PADA MUKHERJEE
FLAT NO 3D
3RD FLOOR PAULAMI APARTMENT
ARRAH
Arra
Burdwan Arrah
Burdwan West Bengal - 713212
8583042932

Generation Date: 27/12/2019



QR Code with Photograph

आपका आधार क्रमांक / Your Aadhaar No. :

5090 9241 9308

VID : 9147 5287 8752 8027

मेरा आधार, मेरी पहचान



भारत सरकार
Government of India



Aloke Kumar Mukherjee
Date of Birth/DOB: 05/01/1960
Male/ MALE

5090 9241 9308

VID : 9147 5287 8752 8027

मेरा आधार, मेरी पहचान





THE NEW INDIA ASSURANCE CO. LTD.

Wholly owned by Government of India

Insurance Cashless e-Card

Employee Name:	Sourav Mahapatra	Policy Holder:	Tata Consultancy Services Ltd
Employee ID:	340754		
Policy Start Date:	01-Apr-2025	Policy End Date:	31-Mar-2026
Room Eligibility 1:	Base Single AC Room for Employee/Spouse/Child		
Room Eligibility 2:	Twin Sharing AC Room for Parent/Parent-in-law		

Beneficiary name	Medi Assist ID	DOB	Relation
Sourav Mahapatra	4009575691	28-Apr-1987	Self
Sangeeta Mahapatra	4016266205	01-Nov-1990	Spouse
Adhiraj Mahapatra	4022312293	26-Jun-2018	First Child
Aloke Kumar Mukherjee	4021570814	05-Jan-1960	Father-in-law

HELPLINE NUMBERS:

24 hours helpline : 040-68213610

- This card is only for identification and is not an authorization to proceed with the treatment or a guarantee for payment.
- In the case of photoless identity cards issued to beneficiaries, acceptable proof of identity such as Aadhar Card/Passport/Driver License/ Ration Card / Voters ID Card / PAN Card should be presented at hospitals.
- This non-transferable identification card is valid at selected Network Hospitals & will enable Card Holder to avail cashless hospitalization only on the basis of preauthorization by Medi Assist.
- For the latest updated hospital list, login to www.mediassisttpa.in.

Important Notes:

- Co-pay has been introduced on all hospitalization claims under TCS India Health Insurance Scheme (HIS), for more details refer the HIS policy document
- Room Rent Eligibility for parents and parent in law for all plan except Platinum Plus and Platinum plan is "Entry Level Twin Sharing AC". Opting higher room category than eligibility will cause proportionate deduction which is applicable to all bill line items and not just the room rent difference. This deduction to be borne by the employee.
- Pre-existing ailments / diseases will not be covered for any newly added parents or parents-in law unless defined under critical illness. However this is not applicable to beneficiaries added within 90 days from the date of joining or marriage or childbirth.

For Cashless Pre-Authorization request/queries please write to cashless@mediassist.in.

Tracking your claim is much simpler and at your fingertips. Access t.mediassist.in

Download our MediAssist app to track/manage your health care needs and do much more - now available in Android and IOS

Medi Assist Insurance TPA Pvt. Ltd.

Tower D, 4th Floor, IBC Knowledge Park, 4/1 Bannerghatta Road, Bhavani Nagar, Bengaluru, Karnataka
560029. CIN: U85199KA1999PTC025676 Website: www.mediassisttpa.in



DEPARTMENT OF GENERAL MEDICINE - CARDIOLOGY

DISCHARGE SUMMARY

Patient Name	: Mr. Alope Kumar Mukherjee	UHID	: 390518
Age/Gender	: 65 Yrs/Male	IP No.	: 25/14029
Admission Date	: 27/06/2025 01:51 PM	Discharge Date	: 03/07/2025
Sponsor	: Medi Assist Insurance TPA Pvt. Ltd	Bed No	: GW-M3-5
Consultant	: Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Secondary Doctor	: Dr. S N Dutta/Dr M R Jena
Address	: FLAT NO : 30, 3RD FLOOR, PAULAMI APARTMENT P.O: - ARRAH P.S : - BIDHANNAGAR , DURGAPUR, WEST BENGAL, INDIA, 713212		

DIAGNOSIS :

- DECOMPENSATED CHRONIC LIVER DISEASE [ICD-11-DB99.0].
- REFRACTORY HYPERKALEMIA CAUSING JUNCTIONAL BRADYCARDIA.
- CHOLELITHIASIS [ICD-11-DC11.Z].
- HYPERTENSION [ICD-11-BA00.Z].
- IRON DEFICIENCY ANAEMIA [ICD-11-3A00.Z]
- HYPOTHYROIDISM [ICD-11-5A00.Z].

CHIEF COMPLAINT AND REASON FOR ADMISSION :

This 65-year-old male, presented with complaints of sudden onset severe generalized weakness for last 1 day with mild chest discomfort and shortness of breath. He was admitted here for evaluation and management.

ALLERGIES : No known drug allergy.

PAST MEDICAL / SURGICAL HISTORY : Hypertension, hypothyroidism, cholelithiasis, DCLD.

FAMILY / SOCIAL HISTORY : Not contributory.

PHYSICAL EXAMINATION :

No pallor, anaemia, icterus, clubbing, cyanosis, edema or lymphadenopathy. No raised JVP.

Vitals: BP: 110/70 mmHg, Pulse: 40/min, RR: 20/min, Temp: 98.2 degree F, SpO2: 98% in RA, CBG: 171 mg/dl.

HEENT: No complaints of headache, change in vision, nose or ear problem or sore throat.

Chest: Bilateral air entry +.

CVS: S1 S2 audible, no murmur.

Abdomen: Soft, bowel sound +.

Extremities: Moving all four limbs on request.

CNS: No neurological deficit.



Patient Name : Mr. Aloke Kumar Mukherjee

UHID

: 390518

INVESTIGATIONS :

Date - 27/06/2025 HEPATITIS B SURFACE ANTIGEN : NON-REACTIVE. HIV 1 & 2 ANTIBODIES SCREENING TEST : NON-REACTIVE. Troponin I (High Sensitive): 29.6 ng/L, NT proBNP: 2323 pg/mL Reactive Protein [CRP]: 3.00 mg/L, PCT (Procalcitonin): 0.304 ng/mL, Sodium (Serum): 122 mmol/L, Potassium (Serum): 7.1 mmol/L, Colour:- Pale Yellow, Volume: 30 mL, Appearance: Clear, pH:- 5.0, Specific Gravity: 1.010, Protein: Negative, Glucose: Negative, Ketone: Negative, Urobilinogen: Negative, Bilirubin: Negative, Nitrite: Negative, Hb / Erythrocytes: Negative, Leukocytes: 0-1/HPF, Epithelial Cells : Occasional/HPF, Crystals [per hpf]: Nil, Casts: Nil, Urine C/S, Bacterial Urine Culture result: No growth after 48 hours of aerobic incubation at 37 degree C.,

Date - 28/06/2025 Hemoglobin: 9.9 g/dL, Red Blood Cell [RBC] Count: $4.05 \times 10^6/\mu\text{L}$, Packed cell volume [PCV]/Haematocrit [HCT]: 33.7 %, MCV : 83.2 fl, MCH: 24.4 pg, MCHC: 29.4 g/dL, RDW CV: 20.3 %, Platelet count: $155 \times 10^3/\mu\text{L}$, Total Leucocyte Count [TLC]: $12.56 \times 10^3/\mu\text{L}$,

Lymphocytes: 9.2 %, Monocytes: 1.8 %, Eosinophils: 0.2 %, Neutrophil-to-Lymphocyte ratio [NLR]: 9.63 ., Urea: 86.7 mg/dL, Serum Creatinine: 1.22 mg/dL, Serum Creatinine: For Confirmation, please check KFT [Test code 32516], eGFR [Test code], UACR [Test code 5323], Uric Acid: 8.58 mg/dL, Sodium (Serum): 129 mmol/L, Potassium (Serum): 6.5 mmol/L, Chloride (Serum): 108 mmol/L, Calcium: 8.73 mg/dL, Phosphorus (Serum): 4.42 mg/dL, Sodium (Serum): 127.3 mmol/L, Potassium (Serum): 6.05 mmol/L,

Date - 29/06/2025 Hemoglobin: 9.4 g/dL, Red Blood Cell [RBC] Count: $3.94 \times 10^6/\mu\text{L}$, Packed cell volume [PCV]/Haematocrit [HCT]: 32.6 %, MCV : 82.7 fl, MCH: 23.9 pg, MCHC: 28.8 g/dL, RDW CV: 19.9 %, Platelet count: $147 \times 10^3/\mu\text{L}$, Total Leucocyte Count [TLC]: $8.83 \times 10^3/\mu\text{L}$,

Lymphocytes: 13 %, Monocytes: 1.6 %, Eosinophils: 0.3 %, Neutrophil-to-Lymphocyte ratio [NLR]: 6.531 ., Urea: 65 mg/dL, Serum Creatinine: 1.04 mg/dL, Serum Creatinine: Please correlate clinically, Uric Acid: 7.88 mg/dL, Sodium (Serum): 131 mmol/L, Potassium (Serum): 6.7 mmol/L, Chloride (Serum): 110 mmol/L, Calcium: 8.63 mg/dL, Phosphorus (Serum): 4.53 mg/dL, Sodium (Serum): 133 mmol/L, Potassium (Serum): 6.2 mmol/L,

Date - 30/06/2025 Sodium (Serum): 134 mmol/L, Potassium (Serum): 5.2 mmol/L, Hemoglobin: 9.5 g/dL, Red Blood Cell [RBC] Count: $3.90 \times 10^6/\mu\text{L}$, Packed cell volume [PCV]/Haematocrit [HCT]: 32.6 %, MCV: 83.6 fl, MCH: 24.4 pg, MCHC: 29.1 g/dL, RDW CV: 19.9 %, Platelet count: $131 \times 10^3/\mu\text{L}$, Total Leucocyte Count [TLC]: $7.82 \times 10^3/\mu\text{L}$, Lymphocytes: 15 %, Monocytes: 1.8 %, Eosinophils: 0.3 %, Neutrophil-to-Lymphocyte ratio [NLR]: 5.507 ., Iron: 19.0 ug/dL, Un-Saturated Iron Binding Capacity: 214.10 ug/dL, Total Iron Binding Capacity [TIBC]: 233.1 ug/dL, Transferrin Saturation: 8.151 %, Ferritin: 50.43 ng/mL, Folic Acid (Serum): 20.00 ng/mL, Folic Acid, serum Decreased folate levels are seen in chronic alcoholism, inadequate dietary intake, malnutrition, megaloblastic anemia. Interference may occur from biotin supplements, methotrexate therapy, etc., Urea: 54.6 mg/dL, Serum Creatinine: 1.13 mg/dL, Serum Creatinine: For Confirmation, please check KFT [Test code 32516], eGFR [Test code], UACR [Test code 5323], Thyroid Stimulating Hormone (TSH)/Thyrotropin: 1.23 uIU/mL, Vitamin B12: 2000 pg/mL, Reticulocyte count: 3.16 %, Lactate Dehydrogenase (LDH): 213.00 U/L,

Direct Coomb's Test result: NEGATIVE, Coombs Test, Direct Antiglobulin Test (DAT), GEL (CAT)

Date - 01/07/2025 Glycosylated Haemoglobin [HbA1c] in %: 4.30 %, Total Cholesterol: 40 mg/dL, Triglyceride: 42 mg/dL, HDL Cholesterol: 24.6 mg/dL, LDL Cholesterol: 11.5 mg/dL, VLDL Cholesterol: 8.4 mg/dL, HDL / LDL Cholesterol Ratio: 2.14 Ratio, Total Cholesterol / HDL Ratio: 1.63 Ratio, Sodium (Serum): 131 mmol/L, Potassium (Serum): 6.1 mmol/L, Urine albumin: 2.31 mg/dL, Urine Creatinine in mg/dL: 55.55 mg/dL,



Patient Name : Mr. Aloke Kumar Mukherjee

UHID

: 390518

Urine albumin Creatinine Ratio [ACR, UACR]: 41.584 ug/mg , Albumin/Creatinine Ratio, Urine Spot (ACR, UACR): For Confirmation, please check KFT [Test code 32516], eGFR [Test code], UACR [Test code 5323], Calcium: 7.78 mg/dL, Potassium (Serum): 5.3 mmol/L,
Date - 02/07/2025 Sodium (Serum): 135.1 mmol/L, Potassium (Serum): 5.00 mmol/L,

RADIOLOGY (as reported):

X-RAY: CHEST AP (27/06/2025): Bronchitic changes.

USG - WHOLE ABDOMEN (28/06/2025): Mild hepatomegaly with coarse echotexture, likely CLD (Suggested - CECT abdomen/ LFT correlation). Cholelithiasis. Dilated PV. Moderate splenomegaly. Minimal ascites.

ECHOCARDIOGRAM WITH DOPPLER (27/06/2025): Concentric LVH. No RWMA. Mild TR. No PAH. Normal LV systolic function. No PAH.

COURSE IN THE HOSPITAL :

This gentleman was admitted with the above-mentioned complaints. He was evaluated clinically and with relevant investigations. He is a known case of hypertension, hypothyroidism, cholelithiasis and DCLD.

After evaluation, the patient was found to have iron deficiency anaemia, junctional bradycardia, dyselectrolytemia, decompensated chronic liver disease, cholelithiasis, hypothyroidism and hypertension.

USG whole abdomen mild hepatomegaly with coarse echotexture, likely CLD (Suggested - CECT abdomen/ LFT correlation). Cholelithiasis. Dilated PV. Moderate splenomegaly. Minimal ascites.

The patient was continuously monitored, managed and treated conservatively with IV antibiotic, s/c insulin therapy, aerosol therapy, PPI, antiemetic, potassium correction therapies, iron supplement and supportive medication.

Opinion of Cardiologist for bradycardia and Gastroenterologist for chronic liver disease and advice was followed accordingly. Cardiologist's advised for holter monitoring, but was refused by the relatives.

Gradually, the patient responded to the treatment and his condition improved. Prognosis and further management plan was discussed with the family members, in details. At present, his condition is stable. He is being discharged with advice to followup in OPD.



Patient Name : Mr. Alope Kumar Mukherjee

UHID

: 390518

MEDICATION ADMINISTERED :

IV-antibiotic

Insulin therapy

Aerosol therapy

PPI

Antiemetic

Sodium and potassium supplement

Iron supplement

CONDITION AT THE TIME OF DISCHARGE :

Hemodynamically stable.

ADVICE AT THE TIME OF DISCHARGE :

ACTIVITIES : Rest for 10 days then as tolerated

DIET : Low salt diet. (< 2 gms salt in 24 hours).

MEDICATION :

-Tab. NUSAM 400 mg (Ademetionine) one tablet twice daily after breakfast and dinner to continue

-Syp. DUPHALAC (Lactulose) 30 ml twice daily after breakfast and dinner to continue

-Tab. FERRISOME (Ferric Pyrophosphate) one tablet once daily after breakfast to continue.

-Tab. THRONORM 75 mcg one tablet once daily before breakfast to continue

-Tab. FEBUTAZ 40 mg (Febuxostat) one tablet once daily after breakfast to continue

-Tab. CILACAR 10 mg (Cilnidipine) one tablet once daily at 8 a.m. to continue

-Cap. PROBOTECH-45 (Lactobacillus) one capsule twice daily after breakfast and dinner for 20 days.

OTHERS :

-Check BP regularly at home [target BP - 110-140/70-90 mmHg] and maintain chart.

FOLLOWUP :

-To attend at the General Medicine OPD after 5 days with CBC, urea, creatinine, sodium, potassium reports and fresh ECG (12 leads) reports.



Patient Name : Mr. Aloke Kumar Mukherjee

UHID

: 390518

WHEN AND HOW TO OBTAIN URGENT CARE :

In case of emergency, such as fever, drug reaction, breathing difficulty, chest pain, abdominal pain, nausea/vomiting, headache, constipation/loose stools, urinary incontinence, etc., dial 8170055555 / 8170066660 for immediate assistance or attend Emergency Department.

* For General Medicine OPD appointment, kindly Call - 8170052847, between 04:00 p.m. - 07:00 p.m. (Mon - Sat).

** FOR SAMPLE COLLECTION FROM HOME, Contact: 8170052815 (HWH), 8170052940 (Bidhannagar), 8170052700 (Asansol), 8170052680 (Dhanbad) **

Dr. Dwipen Kumar Roy,
MBBS, MD (General Medicine)
HOD - General Medicine
Regn. No-41813

Dr. Siddhartha Banerjee,
MBBS, MD (General Medicine)
Senior Consultant - General Medicine
Regn. No- 58286 (KMC)

Dr. Piyusha Ranjan Nayak,
MBBS, DNB
Senior Consultant - General Medicine
Regn No - 82711 (WBMC)

Dr. Taraknath Chattopadhyay,
MBBS, MD (General Medicine), MRCP 1 (UK)
Consultant-General Medicine
Regn No : 71291 WBMC

Dr. Ashok Kumar Parida,
MD (Med.), DNB (Cardio.), MNAMS, FACC, FESC
Sr. Interventional Cardiologist [Reg No: 73445 (WBMC)]

Dr. (Major Retd.) Indranil Dev,
MD (General Medicine), DNB (Cardiology)
Sr. Interventional Cardiologist [Reg No: 54678 (WBMC)]

Dr. Satyendra Nath Dutta,
MD (General Medicine), DM (Cardiology), FSCAI (USA)
Sr. Interventional Cardiologist [Reg No : 64993 (WBMC)]



Discharge summary explained by:

Discharge summary received by:



Tax Invoice / Bill of Supply (IP Bill Summary)

GSTIN : 19AAHCP6873G1ZW

Billed To :	Original	Shipped To (Place Of Supply)
		Healthworld Hospitals, Durgapur
I.P. No. : 25/14029		Bill No. : IPCR26/8977
UHID : 390518		Bill Date : 03/07/2025 12:25 PM
Patient Name : MR. ALOKE KUMAR MUKHERJEE		Consultant : Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay
Gender/Age : Male/65 Yr 5 Mth 28 Days		D.O.A : 27/06/2025 13:51
Contact No : 8583042932		D.O.D : 03/07/2025 12:23
Address : FLAT NO : 30, 3RD FLOOR, PAULAMI APARTMENT P.O: - ARRAH P.S : - BIDHANNAGAR , DURGAPUR, WEST BENGAL		Bed No/Ward : GW-M3-5/General Ward Male - 3 6F
Payer : Medi Assist Insurance TPA Pvt. Ltd		Billing Category : General Ward Bed **
Sponsor : Medi Assist Insurance TPA Pvt. Ltd		SAC Code : 9993
Payer GSTIN :		
Payer Address : Kolkata KOLKATA WEST BENGAL		

S#	Particulars	Gross Amt	Disc Amt	Net Amt	Pat Amt	Payer Amt
1	Bed Charges	32000.00	3200.00	28800.00	0.00	28800.00
2	Consultation IP	30500.00	0.00	30500.00	0.00	30500.00
3	Investigation - Laboratory	37070.00	3707.00	33363.00	0.00	33363.00
4	Investigation - Radiology	1800.00	180.00	1620.00	0.00	1620.00
5	Investigation - Point of Care	3990.00	399.00	3591.00	0.00	3591.00
6	Investigation - Cardiology	2000.00	200.00	1800.00	0.00	1800.00
7	Cardiology	400.00	40.00	360.00	0.00	360.00
8	Medicines,Medical and Surgical Items	24864.11	1886.51	22977.60	0.00	22977.60
9	Consumables	4441.08	0.00	4441.08	0.00	4441.08
10	Equipment Charges	2100.00	210.00	1890.00	0.00	1890.00
		139165.19	9822.5100	129342.68	0.00	129342.68

Bill Not Settled

Gross Amount

139165.19

Agreed MOAMOU Discount (-)

9823.00

Net Amount

129343.00

Amount Received Rs. (-)

18285.00

Amount To Be Received (Rs.)

111058.00

Net Amount In Words:(Rs.) One Lakh Twenty Nine Thousand Three Hundred Forty Three only .

Advance/Payment Details

Receipt/Ref no	Receipt/Ref Date	Received/Ref Amt	Adjusted Amount	Mode
AD26/11738(Settled)	03/07/2025 12:23	18285.00	18285.00	Credit Card,18285.00

Patient's/Attendant's Signature

A Unit Of Parashmani Medical Centre Pvt Ltd

CIN :- U85100WB2014PTC201736

GSTIN :- 19AAHCP6873G1ZW

Bank Details :

Bank Name :- PUNJAB NATIONAL BANK

Bank A/C No. :- 03041132000317

IFSC Code :- PUNB0030410

Branch :- Durgapur Nachan Road

E. & O. E.

For Healthworld Hospitals

Authorised Signatory

In-Patient Bill (Details)

I.P. No. : 25/14029	Original	Bill No. : IPCR26/8927
UHID : 390518		Bill Date : 03/07/2025 12:25 PM
Patient Name : Mr. Aloke Kumar Mukherjee		Consultant : Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay
Gender/Age : Male/65 Yr 5 Mth 28 Days		Adm. Category : Critical Care Bed **
Contact No : 8583042932		D.O.A : 27/06/2025 13:51
Address : FLAT NO : 30, 3RD FLOOR, PAULAMI APARTMENT P.O: - ARRAH P.S : - BIDHANNAGAR DURGAPUR		D.O.D : 03/07/2025 12:23
Payer : Medi Assist Insurance TPA Pvt. Ltd		Bed No/Ward : GW-M3-5/General Ward Male - 3 6F
Sponsor : Medi Assist Insurance TPA Pvt. Ltd		Dis. Category : General Ward Bed **
Payer GSTIN :		SAC Code : 9993
Payer Address : Kolkata KOLKATA WEST BENGAL		

Date	Particulars	Rate	Qty	Disc Amt	Amount
Bed Charges					
27/06/2025	Critical Care Bed **	8000.00	1.00	800.00	7200.00
28/06/2025	Critical Care Bed **	8000.00	1.00	800.00	7200.00
29/06/2025	Critical Care Bed **	8000.00	1.00	800.00	7200.00
30/06/2025	General Ward Bed **	2000.00	1.00	200.00	1800.00
01/07/2025	General Ward Bed **	2000.00	1.00	200.00	1800.00
02/07/2025	General Ward Bed **	2000.00	1.00	200.00	1800.00
03/07/2025	General Ward Bed **	2000.00	1.00	200.00	1800.00
Total for Bed Charges					28800.00
Consultation IP					
27/06/2025	EVENING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	2000.00	1.00	0.00	2000.00
27/06/2025	EVENING VISIT (Dr. D Mazumder / Dr P Pallavi)	2500.00	1.00	0.00	2500.00
28/06/2025	MORNING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	2000.00	1.00	0.00	2000.00
28/06/2025	MORNING VISIT (Dr. S N Dutta/Dr M R Jena)	2500.00	1.00	0.00	2500.00
28/06/2025	EVENING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	2000.00	1.00	0.00	2000.00
29/06/2025	MORNING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	2000.00	1.00	0.00	2000.00
29/06/2025	EVENING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	2000.00	1.00	0.00	2000.00
29/06/2025	MORNING VISIT (Dr. S N Dutta/Dr M R Jena)	2500.00	1.00	0.00	2500.00
30/06/2025	MORNING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00
30/06/2025	MORNING VISIT (Dr. Mrunmaya Ricky Jena)	1500.00	1.00	0.00	1500.00
30/06/2025	EVENING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00
30/06/2025	MORNING VISIT (Dr. S N Dutta/Dr M R Jena)	1500.00	1.00	0.00	1500.00
01/07/2025	MORNING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00
01/07/2025	EVENING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00
01/07/2025	MORNING VISIT (Dr. S N Dutta/Dr M R Jena)	1500.00	1.00	0.00	1500.00
02/07/2025	MORNING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00
02/07/2025	MORNING VISIT (Dr. S N Dutta/Dr M R Jena)	1500.00	1.00	0.00	1500.00
02/07/2025	EVENING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00
03/07/2025	MORNING VISIT (Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay)	1000.00	1.00	0.00	1000.00

I.P. No. : 25/14029 Bill No. : IPCR26/8927 UHID : 390518 Patient Name : Mr. Aloke Kumar Mukherjee

Printed By: abhinaba Prepared By: Samagata Halder Page No : 1 of 7 Print Date & Time: 03/07/2025 07:10 PM

		Total for Consultation IP			30500.00
Investigation - Cardiology					
27/06/2025	ECHOCARDIOGRAM WITH DOPPLER	2000.00	1.00	200.00	1800.00
Total for Investigation - Cardiology					1800.00
Investigation - Point of Care					
27/06/2025	ABG (Blood Gas)	600.00	1.00	60.00	540.00
27/06/2025	CBG (Capillary Blood Glucose)	70.00	1.00	7.00	63.00
27/06/2025	Troponin T (Card Test)	1000.00	1.00	100.00	900.00
27/06/2025	CBG (Capillary Blood Glucose)	70.00	1.00	7.00	63.00
28/06/2025	ABG (Blood Gas)	600.00	1.00	60.00	540.00
28/06/2025	CBG (Capillary Blood Glucose)	70.00	3.00	21.00	189.00
29/06/2025	ABG (Blood Gas)	600.00	1.00	60.00	540.00
29/06/2025	CBG (Capillary Blood Glucose)	70.00	3.00	21.00	189.00
30/06/2025	CBG (Capillary Blood Glucose)	70.00	3.00	21.00	189.00
01/07/2025	CBG (Capillary Blood Glucose)	70.00	3.00	21.00	189.00
02/07/2025	CBG (Capillary Blood Glucose)	70.00	3.00	21.00	189.00
Total for Investigation - Point of Care					3591.00
Investigation - Laboratory					
27/06/2025	Serology Rapid Screening (HCV, HBSAG & HIV 1&2)	1000.00	1.00	100.00	900.00
27/06/2025	Troponin - I (HS), serum	1625.00	1.00	162.50	1462.50
27/06/2025	NT-proBNP (N Terminal B Type Natriuretic Peptide)	2250.00	1.00	225.00	2025.00
27/06/2025	C Reactive Protein (CRP)	1000.00	1.00	100.00	900.00
27/06/2025	Procalcitonin	4000.00	1.00	400.00	3600.00
27/06/2025	Serum Sodium	450.00	1.00	45.00	405.00
27/06/2025	Serum Potassium	450.00	1.00	45.00	405.00
27/06/2025	Urine Routine Examination	150.00	1.00	15.00	135.00
27/06/2025	Urine C/S, Bacterial	1000.00	1.00	100.00	900.00
28/06/2025	Renal Profile Basic(CBC,Ur, Cr, Elec, Cal, Phos, UA)	1710.00	1.00	171.00	1539.00
28/06/2025	Serum Sodium	450.00	1.00	45.00	405.00
28/06/2025	Serum Potassium	450.00	1.00	45.00	405.00
29/06/2025	Renal Profile Basic(CBC,Ur, Cr, Elec, Cal, Phos, UA)	1710.00	1.00	171.00	1539.00
29/06/2025	Serum Sodium	450.00	1.00	45.00	405.00
29/06/2025	Serum Potassium	450.00	1.00	45.00	405.00
30/06/2025	ANAEMIA PANEL (CBC, Ab Hb variants, Iron studies (Iron, TIBC, Transferrin Sat., Ferritin), vit B12,fol,urea,creat, TSH,Reticulocyte Count,LDH,DCT,PBS)	7700.00	1.00	770.00	6930.00
30/06/2025	Serum Sodium	450.00	1.00	45.00	405.00
30/06/2025	Serum Potassium	450.00	1.00	45.00	405.00
01/07/2025	Glycosylated Haemoglobin (HbA1c) blood	850.00	1.00	85.00	765.00
01/07/2025	Lipid Profile Basic [PANEL] (T.Chol,Trig,HDL,LDL,VLDL) (Fasting)	750.00	1.00	75.00	675.00
01/07/2025	Serum Sodium	450.00	1.00	45.00	405.00
01/07/2025	Serum Potassium	450.00	1.00	45.00	405.00
01/07/2025	Albumin/Creatinine Ratio, Urine Spot (ACR, UACR)	500.00	1.00	50.00	450.00
01/07/2025	Serum Calcium (Total)	200.00	1.00	20.00	180.00
01/07/2025	Serum Potassium	450.00	1.00	45.00	405.00
02/07/2025	Serum Sodium	450.00	1.00	45.00	405.00
02/07/2025	Serum Potassium	450.00	1.00	45.00	405.00
02/07/2025	ACTH (Adrenocorticotrophic Hormone) (TEST OUTSOURCED)(M)	1875.00	1.00	187.50	1687.50
02/07/2025	Aldosterone /renin ratio plasma, aldosterone /PRA ratio plasma (TEST OUTSOURCED)(M)	4900.00	1.00	490.00	4410.00
Total for Investigation - Laboratory					33363.00
Investigation - Radiology					
27/06/2025	X-RAY: CHEST AP	300.00	1.00	30.00	270.00
28/06/2025	USG - BEDSIDE	1500.00	1.00	150.00	1350.00
Total for Investigation - Radiology					1620.00
Cardiology					
27/06/2025	ECG	200.00	1.00	20.00	180.00
28/06/2025	ECG	200.00	1.00	20.00	180.00

		Total for Cardiology			360.00
Medicines,Medical and Surgical Items					
27/06/2025	CARDIVAS 3.125MG TAB(15"S) [B.No:-E-SIG0848A] [ExpDate:-31/03/2028]	78.00	1.00	7.80	70.20
27/06/2025	DUPHALAC 250 ML SYP [B.No:-E-763157D7] [ExpDate:-30/03/2027]	328.12	1.00	32.81	295.31
27/06/2025	PARIDEM 40MG INJ [B.No:-E-EP06BU4022] [ExpDate:-30/10/2026]	56.50	1.00	5.65	50.85
27/06/2025	VOMIKIND 2ML INJ [B.No:-E-E51X183] [ExpDate:- 30/10/2027]	13.35	3.00	4.01	36.04
27/06/2025	NS 3%100ML(DENIS) [B.No:-E-2503042004] [ExpDate:-30/12/2027]	173.00	3.00	51.90	467.10
27/06/2025	SYRINGE 50ML - ROMSONS [B.No:-K2SD020174] [ExpDate:-31/03/2030]	89.00	1.00	0.00	89.00
27/06/2025	PM-O-LINE 200 CM - MF - ROMSONS [B.No:- K2SB011045] [ExpDate:-31/01/2030]	425.00	1.00	0.00	425.00
27/06/2025	MEDINEXT-PLUS 1.125GM INJ [B.No:-E-CI24251A] [ExpDate:-30/12/2026]	799.00	2.00	159.80	1438.20
27/06/2025	THYRONORM 75 MCG TAB (100"S) [B.No:-E- TMF25060] [ExpDate:-31/03/2027]	196.24	1.00	19.62	176.62
27/06/2025	HUMAN ACTRAPID 40IU 10ML INJ [B.No:-E-B70895] [ExpDate:-30/06/2027]	178.29	1.00	17.83	160.46
27/06/2025	ASTHALIN RESPULES 2.5ML [B.No:-E-RL81177] [ExpDate:-30/10/2027]	8.54	4.00	3.42	30.74
27/06/2025	FERISOME 30MG TAB (15"S) [B.No:-E-FRSMGC009] [ExpDate:-30/03/2027]	348.00	1.00	34.80	313.20
27/06/2025	K-BIND POWDER 15G [B.No:-E-BRB1027] [ExpDate:- 31/12/2026]	159.00	3.00	47.70	429.30
27/06/2025	IV SET - ROMSONS [B.No:-K2SB020715] [ExpDate:- 31/01/2030]	205.00	1.00	0.00	205.00
27/06/2025	PEDIAADRIJ SET - ROMSONS [B.No:-K2SC020844] [ExpDate:-28/02/2030]	307.00	1.00	0.00	307.00
27/06/2025	ECG ELECTRODE - ADLISC [B.No:-0520525] [ExpDate:-30/04/2027]	33.60	5.00	0.00	168.00
27/06/2025	SYRINGE 10ML - ROMSONS [B.No:-K2SE020346] [ExpDate:-30/04/2030]	39.50	3.00	0.00	118.50
27/06/2025	SYRINGE 2 ML - ROMSONS [B.No:-K2SD020106] [ExpDate:-31/03/2030]	16.30	3.00	0.00	48.90
27/06/2025	SYRINGE 20ML - ROMSONS [B.No:-K2SC020528] [ExpDate:-28/02/2030]	51.00	2.00	0.00	102.00
27/06/2025	SYRINGE 5ML - ROMSONS [B.No:-K2SC020980] [ExpDate:-28/02/2030]	21.00	3.00	0.00	63.00
27/06/2025	Iv Cannula (A) 27G - Romsons [B.No:-G2SA010973] [ExpDate:-31/12/2029]	198.00	1.00	0.00	198.00
27/06/2025	Iv Cannula (A) 18G - Romsons [B.No:-K22081708] [ExpDate:-31/07/2027]	176.00	1.00	0.00	176.00
27/06/2025	VEIN-O-LINE 10 CM - ROMSONS [B.No:-K2SB010941] [ExpDate:-30/01/2030]	360.00	1.00	0.00	360.00
27/06/2025	NEEDLE 18G - DISPOVAN [B.No:-065340] [ExpDate:- 31/01/2030]	3.30	3.00	0.00	9.90
27/06/2025	HEPLOCK 10ML INJ [B.No:-E-G1020018] [ExpDate:- 30/08/2027]	50.00	1.00	5.00	45.00
27/06/2025	IV CANNULA (A) 20G - ROMSONS [B.No:- G2SB010817] [ExpDate:-31/01/2030]	198.00	1.00	0.00	198.00
27/06/2025	VEIN-O-LINE 10 CM - ROMSONS [B.No:-K2SB010941] [ExpDate:-30/01/2030]	360.00	1.00	0.00	360.00
27/06/2025	PARIDEM 40MG INJ [B.No:-E-EP06BU4022] [ExpDate:-30/10/2026]	56.50	1.00	5.65	50.85
27/06/2025	VOMIKIND 2ML INJ [B.No:-E-E51X183] [ExpDate:- 30/10/2027]	13.35	1.00	1.34	12.01
27/06/2025	GLUCI 10ML INJECTION [B.No:-E-KP1169815] [ExpDate:-31/12/2026]	91.50	1.00	9.15	82.35
27/06/2025	D 25% 100ML IV [B.No:-E-2503045003] [ExpDate:- 31/03/2026]	22.40	1.00	2.24	20.16
27/06/2025	NS 3%100ML(DENIS) [B.No:-E-2503042004] [ExpDate:-30/12/2027]	173.00	1.00	17.30	155.70
27/06/2025	ASTHALIN RESPULES 2.5ML [B.No:-E-RL81177] [ExpDate:-30/10/2027]	8.54	12.00	10.25	92.23
27/06/2025	NEBULIZER MASK (A) - ROMSONS [B.No:- K2SC040525] [ExpDate:-28/02/2030]	690.00	1.00	0.00	690.00

27/06/2025	SODIUM BICARBONATE 25ML INJ [B.No:-E-S268] [ExpDate:-30/09/2026]	34.00	4.00	13.60	122.40
27/06/2025	INSULIN SYRINGE 40 U DISPOVAN - HMD [B.No:- 446014AG21] [ExpDate:-31/10/2029]	12.00	1.00	0.00	12.00
27/06/2025	ECG ELECTRODE - ADLISC [B.No:-0530525] [ExpDate:-30/04/2027]	33.60	5.00	0.00	168.00
27/06/2025	SYRINGE 2 ML - ROMSONS [B.No:-K25D020106] [ExpDate:-30/03/2030]	16.30	2.00	0.00	32.60
27/06/2025	SYRINGE 20ML - ROMSONS [B.No:-K25E020367] [ExpDate:-30/04/2030]	55.70	2.00	0.00	111.40
27/06/2025	SYRINGE 5ML - ROMSONS [B.No:-K25C020980] [ExpDate:-28/02/2030]	21.00	2.00	0.00	42.00
27/06/2025	GLUCI 10ML INJECTION [B.No:-E-KP1169801] [ExpDate:-31/10/2026]	91.50	3.00	27.45	247.05
27/06/2025	UDOSIS 500MG TAB (10"S) [B.No:-E-UDOR029] [ExpDate:-31/12/2026]	41.00	1.00	4.10	36.90
27/06/2025	ADESAM 400MG TAB(10"S) [B.No:-E-CPC0003] [ExpDate:-30/06/2026]	925.00	1.00	92.50	832.50
27/06/2025	SYRINGE 5ML - ROMSONS [B.No:-K25C020980] [ExpDate:-28/02/2030]	21.00	3.00	0.00	63.00
27/06/2025	D 25% 100ML INJ (DENTIS) [B.No:-E-2503045002] [ExpDate:-28/02/2028]	22.40	2.00	4.48	40.32
28/06/2025	HEPARINE 5000IU 1000IU/ML INJ [B.No:-E- G1000032] [ExpDate:-31/12/2027]	115.00	1.00	11.50	103.50
28/06/2025	FEBUTAZ 40MG TAB(15"S) [B.No:-E-SIG0244A] [ExpDate:-30/06/2027]	246.00	1.00	24.60	221.40
28/06/2025	RESODIM 15 MG TAB (4"S) [B.No:-E-BRE08067A] [ExpDate:-30/07/2026]	803.20	1.00	80.32	722.88
28/06/2025	MEDINEXT-PLUS 1.125GM INJ [B.No:-E-CI24251A] [ExpDate:-30/12/2026]	799.00	2.00	159.80	1438.20
28/06/2025	GLUCI 10ML INJECTION [B.No:-E-KP1169801] [ExpDate:-31/10/2026]	91.50	3.00	27.45	247.05
28/06/2025	D 25% 100ML IV [B.No:-E-2503045003] [ExpDate:- 31/03/2026]	22.40	2.00	4.48	40.32
28/06/2025	ASTHALIN RESPULES 2.5ML [B.No:-E-4L81177] [ExpDate:-30/10/2027]	8.54	4.00	3.42	30.74
28/06/2025	PANTOCID 40MG INJ [B.No:-E-NPB00022] [ExpDate:- 31/12/2026]	56.50	1.00	5.65	50.85
28/06/2025	VOMIKIND 2ML INJ [B.No:-E-ES1X183] [ExpDate:- 30/10/2027]	13.35	3.00	4.01	36.04
28/06/2025	ECG ELECTRODE - ADLISC [B.No:-0520525] [ExpDate:-30/04/2027]	33.60	6.00	0.00	201.60
28/06/2025	SYRINGE 10ML - ROMSONS [B.No:-K25E020402] [ExpDate:-30/04/2030]	39.50	4.00	0.00	158.00
28/06/2025	SYRINGE 5ML - ROMSONS [B.No:-K25C020980] [ExpDate:-28/02/2030]	21.00	5.00	0.00	105.00
28/06/2025	PROBOTECH 90 TAB (10"S) [B.No:-E-SC240424] [ExpDate:-28/02/2026]	1790.00	1.00	179.00	1611.00
29/06/2025	MEDINEXT-PLUS 1.125GM INJ [B.No:-E-CI24251A] [ExpDate:-30/12/2026]	799.00	2.00	159.80	1438.20
29/06/2025	GLUCI 10ML INJECTION [B.No:-E-KP1169801] [ExpDate:-31/10/2026]	91.50	3.00	27.45	247.05
29/06/2025	D 25% 100ML IV [B.No:-E-2503045003] [ExpDate:- 31/03/2026]	22.40	3.00	6.72	60.48
29/06/2025	ASTHALIN RESPULES 2.5ML [B.No:-E-4L81177] [ExpDate:-30/10/2027]	8.54	4.00	3.42	30.74
29/06/2025	PANTOCID 40MG INJ [B.No:-E-NPB00022] [ExpDate:- 31/12/2026]	56.50	1.00	5.65	50.85
29/06/2025	VOMIKIND 2ML INJ [B.No:-E-ES1X183] [ExpDate:- 30/10/2027]	13.35	4.00	5.34	48.06
29/06/2025	SEEBIND 15GM SACHET [B.No:-E-SBND035] [ExpDate:-30/12/2026]	167.00	1.00	16.70	150.30
29/06/2025	SOBISIS 500MG TAB(15"S) [B.No:-E-D224779] [ExpDate:-30/07/2027]	67.00	1.00	6.70	60.30
29/06/2025	THROMBOPHOB GEL 30GM [B.No:-E-IA0050A] [ExpDate:-30/03/2028]	342.53	1.00	34.25	308.28
29/06/2025	SYRINGE 10ML - ROMSONS [B.No:-K25E020346] [ExpDate:-30/04/2030]	39.50	3.00	0.00	118.50
29/06/2025	SYRINGE 5ML - ROMSONS [B.No:-K25C020980] [ExpDate:-28/02/2030]	21.00	3.00	0.00	63.00

I.P. No. : 25/14029 Bill No. : IPCR26/8927 UHID : 390518 Patient Name : Mr. Aloke Kumar Mukherjee

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29/06/2025	ECG ELECTRODE - ADLISC [B.No:-0520525] [ExpDate:-30/04/2027]	33.60	5.00	0.00	168.00
29/06/2025	Iv Cannula (A) 18G - Romsons [B.No:-K22081708] [ExpDate:-31/07/2027]	176.00	1.00	0.00	176.00
29/06/2025	NUTRIKA TABLETS (10'S) [B.No:-E-SPT50069] [ExpDate:-30/08/2026]	330.00	1.00	33.00	297.00
30/06/2025	MEDINEXT-PLUS 1.125GM INJ [B.No:-E-CI24251A] [ExpDate:-30/12/2026]	799.00	2.00	159.80	1438.20
30/06/2025	PANTOCID 40MG INJ [B.No:-E-NP000022] [ExpDate:- 31/12/2026]	56.50	1.00	5.65	50.85
30/06/2025	VOMIKIND 2ML INJ [B.No:-E-E51X183] [ExpDate:- 30/10/2027]	13.35	3.00	4.01	36.04
30/06/2025	SEEBIND 15GM SACHET [B.No:-E-SBND035] [ExpDate:-30/12/2026]	167.00	1.00	16.70	150.30
30/06/2025	SYRINGE 10ML - ROMSONS [B.No:-K25E020402] [ExpDate:-30/04/2030]	39.50	5.00	0.00	197.50
30/06/2025	SYRINGE 20ML - ROMSONS [B.No:-K25C020528] [ExpDate:-28/02/2030]	51.00	1.00	0.00	51.00
30/06/2025	SYRINGE 5ML - ROMSONS [B.No:-K25C020980] [ExpDate:-28/02/2030]	21.00	5.00	0.00	105.00
01/07/2025	ASTHALIN RESPULES 2.5ML [B.No:-E-4LB1177] [ExpDate:-30/10/2027]	8.54	3.00	2.56	73.06
01/07/2025	D 25% 100ML IV [B.No:-E-2503045003] [ExpDate:- 31/03/2026]	22.40	3.00	6.72	60.48
01/07/2025	GLUCI 10ML INJECTION [B.No:-E-KP1169801] [ExpDate:-31/10/2026]	91.50	2.00	18.30	164.70
01/07/2025	MEDINEXT-PLUS 1.125GM INJ [B.No:-E-CI24251A] [ExpDate:-30/12/2026]	799.00	2.00	159.80	1438.20
01/07/2025	PANTOCID 40MG INJ [B.No:-E-NP000022] [ExpDate:- 31/12/2026]	56.50	1.00	5.65	50.85
01/07/2025	VOMIKIND 2ML INJ [B.No:-E-E51X183] [ExpDate:- 30/10/2027]	13.35	3.00	4.01	36.04
01/07/2025	SEEBIND 15GM SACHET [B.No:-E-SBND035] [ExpDate:-30/12/2026]	167.00	1.00	16.70	150.30
01/07/2025	SYRINGE 10ML - ROMSONS [B.No:-K25E020402] [ExpDate:-30/04/2030]	39.50	5.00	0.00	197.50
01/07/2025	SYRINGE 5ML - ROMSONS [B.No:-K25C020980] [ExpDate:-28/02/2030]	21.00	5.00	0.00	105.00
01/07/2025	ADESAM 400MG TAB(10'S) [B.No:-E-CPC0015] [ExpDate:-30/06/2026]	925.00	1.00	92.50	832.50
01/07/2025	IV CANNULA (A) 20G - ROMSONS [B.No:- G25R010590] [ExpDate:-31/01/2030]	198.00	1.00	0.00	198.00
01/07/2025	NEEDLE 18G - DISPOVAN [B.No:-065340] [ExpDate:- 31/01/2030]	3.30	3.00	0.00	9.90
02/07/2025	SEEBIND 15GM SACHET [B.No:-E-SBND035] [ExpDate:-30/12/2026]	167.00	3.00	50.10	950.90
02/07/2025	IV CANNULA (A) 20G - ROMSONS [B.No:- G25E010441] [ExpDate:-30/04/2030]	198.00	1.00	0.00	198.00
03/07/2025	SEEBIND 15GM SACHET [B.No:-E-SBND035] [ExpDate:-30/12/2026]	167.00	1.00	(16.70)	150.30
03/07/2025	CARDIVAS 3.125MG TAB(15'S) [B.No:-E-SIG0848A] [ExpDate:-31/03/2028]	78.00	1.00	(7.80)	70.20
03/07/2025	GLUCI 10ML INJECTION [B.No:-E-KP1169801] [ExpDate:-31/10/2026]	91.50	1.00	(9.15)	82.35
Total for Medicines,Medical and Surgical Items					22977.60
Consumables					
27/06/2025	PACKAGED DRINKING WATER 1 LTR - AQUE BOURNE [B.No:-NCC900] [ExpDate:-01/12/2025]	20.00	1.00	0.00	20.00
27/06/2025	OUNCE GLASS - LOCAL [B.No:-127] [ExpDate:- 08/04/2030]	25.00	1.00	0.00	25.00
27/06/2025	EXAMINATION GLOVES [B.No:-C25124] [ExpDate:- 30/04/2029]	12.00	22.00	0.00	264.00
27/06/2025	COTTON 100 GM - MEDICOT [B.No:-0628] [ExpDate:-31/05/2028]	50.00	1.00	0.00	50.00
27/06/2025	FACE MASK 3 PLY - ST [B.No:-1394] [ExpDate:- 14/06/2030]	8.00	3.00	0.00	24.00
27/06/2025	NURSES CAP - SWS [B.No:-1141] [ExpDate:- 03/06/2030]	8.00	3.00	0.00	24.00
27/06/2025	SURGEONS CAP (SWS) [B.No:-157] [ExpDate:- 08/04/2030]	8.00	3.00	0.00	24.00

27/06/2025	TOOTH BRUSH - COLGATE [B.No:-0078] [ExpDate:-21/05/2028]	20.00	1.00	0.00	20.00
27/06/2025	TOOTH PASTE - COLGATE [B.No:-25] [ExpDate:-10/04/2030]	5.00	1.00	0.00	5.00
27/06/2025	MICROSHIELD HANDRUB 100 ML - SCHULKE [B.No:-FMSHR25041] [ExpDate:-30/03/2027]	204.00	1.00	0.00	204.00
27/06/2025	STERILE SURGICAL GLOVES 7.0 - SURGICARE [B.No:-25B1003] [ExpDate:-31/01/2030]	97.00	2.00	0.00	194.00
27/06/2025	IV 3000 7CMX9CM (REF-4006) - SMITH & NEPHEW [B.No:-202434] [ExpDate:-31/08/2027]	135.52	2.00	0.00	271.04
27/06/2025	URINE POT [B.No:-25] [ExpDate:-10/04/2030]	155.00	1.00	0.00	155.00
27/06/2025	DIGITAL THERMOMETER - MICROGENE [B.No:-MT1027-B] [ExpDate:-06/06/2030]	206.00	1.00	0.00	206.00
27/06/2025	COMB - LOCAL [B.No:-38] [ExpDate:-19/06/2030]	10.00	1.00	0.00	10.00
27/06/2025	IV 3000 7CMX9CM (REF-4006) - SMITH & NEPHEW [B.No:-202434] [ExpDate:-31/08/2027]	135.52	1.00	0.00	135.52
27/06/2025	GAMUEE PAD 5"X6" [B.No:-804] [ExpDate:-15/05/2028]	30.00	2.00	0.00	60.00
27/06/2025	EXAMINATION GLOVES [B.No:-C25124] [ExpDate:-30/04/2029]	12.00	12.00	0.00	144.00
28/06/2025	UNDERPAD [B.No:-1543] [ExpDate:-23/06/2030]	159.00	2.00	0.00	318.00
28/06/2025	EXAMINATION GLOVES [B.No:-C25124] [ExpDate:-30/04/2029]	12.00	10.00	0.00	120.00
28/06/2025	FACE MASK 3 PLY - ST [B.No:-1394] [ExpDate:-14/06/2030]	8.00	3.00	0.00	24.00
28/06/2025	NURSES CAP - SWS [B.No:-1141] [ExpDate:-03/06/2030]	8.00	3.00	0.00	24.00
29/06/2025	EXAMINATION GLOVES [B.No:-1497] [ExpDate:-27/06/2028]	12.00	10.00	0.00	120.00
29/06/2025	FACE MASK 3 PLY - ST [B.No:-1394] [ExpDate:-14/06/2030]	8.00	3.00	0.00	24.00
29/06/2025	GAUZE SWAB 6.5X6.5 (1X2 PC) [B.No:-1237] [ExpDate:-07/06/2030]	10.00	4.00	0.00	40.00
29/06/2025	IV 3000 7CMX9CM (REF-4006) - SMITH & NEPHEW [B.No:-202434] [ExpDate:-31/08/2027]	135.52	1.00	0.00	135.52
29/06/2025	COTTON 100 GM - MED1-COT [B.No:-0628] [ExpDate:-31/05/2028]	50.00	1.00	0.00	50.00
30/06/2025	EXAMINATION GLOVES [B.No:-1497] [ExpDate:-27/06/2028]	12.00	10.00	0.00	120.00
30/06/2025	PAPER GLOVES WITH GRIP - PREMIUM / PETAL [B.No:-1295] [ExpDate:-09/06/2030]	36.00	4.00	0.00	144.00
30/06/2025	PLASTIC APRON - PREMIUM [B.No:-0525] [ExpDate:-30/04/2028]	45.00	1.00	0.00	45.00
30/06/2025	UNDERPAD [B.No:-1543] [ExpDate:-23/06/2030]	159.00	4.00	0.00	636.00
30/06/2025	STERILE SURGICAL GLOVES 7.0 - SURGICARE [B.No:-25B1003] [ExpDate:-31/01/2030]	97.00	1.00	0.00	97.00
01/07/2025	EXAMINATION GLOVES [B.No:-1497] [ExpDate:-27/06/2028]	12.00	10.00	0.00	120.00
01/07/2025	LEUKOMED IV FILM 7X9 CM (REF-72390-05) - BSN [B.No:-43311832] [ExpDate:-30/04/2029]	234.00	1.00	0.00	234.00
02/07/2025	LEUKOMED IV FILM 7X9 CM (REF-72390-05) - BSN [B.No:-43311832] [ExpDate:-30/04/2029]	234.00	1.00	0.00	234.00
02/07/2025	EXAMINATION GLOVES [B.No:-1497] [ExpDate:-27/06/2028]	12.00	10.00	0.00	120.00

Total for Consumables 4441.08

Equipment Charges

27/06/2025	Nebulization (1 Time)	100.00	1.00	10.00	90.00
28/06/2025	Nebulization (1 Time)	100.00	4.00	40.00	360.00
29/06/2025	Nebulization (1 Time)	100.00	4.00	40.00	360.00
30/06/2025	Nebulization (1 Time)	100.00	4.00	40.00	360.00
01/07/2025	Nebulization (1 Time)	100.00	4.00	40.00	360.00
02/07/2025	Nebulization (1 Time)	100.00	4.00	40.00	360.00

Total for Equipment Charges 1890.00

Bill Not Settled

Total Amount	139165.10
Discount Amount (-)	9822.51
Net Amount	129342.68

I.P. No. : 25/14029 Bill No. : IPCR26/8927 UHID : 390518 Patient Name : Mr. Alok Kumar Mukherjee

Printed By: abhinaba Prepared By: Samagata Halder Page No : 6 of 7 Print Date & Time: 02/07/2025 07:10 PM

Amount Received Rs. (-) 18285.00
Amount To Be Received (Rs.) 111058.00

Advance/Payment Details

Receipt/Ref no	Receipt/Ref Date	Received/Ref Amt	Adjusted Amount	Mode
AD26/11738(Settled)	03/07/2025 12:23	18285.00	18285.00	Credit Card,18285.00

Patient's /Attendant's Signature

Authorised Signatory





DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740018
UHID	390518	Sample Date	27/06/2025 2:50PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 3:42PM
Bed No/Ward	ICU-10-5F	Report Dates	27/06/2025 5:33PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

NT-proBNP (N Terminal B Type Natriuretic Peptide)

Sample: Serum

NT proBNP **2323** pg/mL 0 - 125 EQ/A

This is an automated quantitative test for determination of N terminal fragment of B-type natriuretic peptide in human serum or plasma (lithium heparin). The test is used as an aid in the diagnosis of suspected congestive heart failure. The natriuretic peptides, ANP (atrial natriuretic peptide), BNP and C-type peptide, constitute a family of essentially cardiac polypeptides which possess the properties of natriuresis, diuresis, and vasodilation. NT-proBNP assay helps to differentiate between cardiac and pulmonary dyspnea, predicts mortality in patients presenting with acute coronary syndrome. Variations in NT-proBNP concentration may be useful to monitor the efficacy of therapy for left ventricular dysfunction. Renal dysfunction is a potential cause of elevated natriuretic peptides concentrations in the absence of left ventricular dysfunction.

C Reactive Protein (CRP)

Sample: Serum

C Reactive Protein [CRP] **3.00** mg/L 0 - 5 Turbidimetry

End Of Report

Devashis Mandal

Dr. Devashis Mandal

MBBS, MD (Pathology), Consultant - Lab Services, Reg No: 66456 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740018
UHID	390518	Sample Date	27/06/2025 2:50PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 3:42PM
Bed No/Ward	ICU-10-5F	Report Dates	27/06/2025 5:33PM
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Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

PCT (Procalcitonin) 0.304 ng/mL 0 - 1 Sample: Serum

Procalcitonin (PCT) is secreted in response to proinflammatory stimulation, particularly bacterial stimulation. PCT can be elevated in non-infectious causes like- initial days after a major trauma, major surgical intervention, burns, treatment with antibodies and other drugs stimulating the release of pro-inflammatory cytokines, small cell lung cancer, medullary C-cell carcinoma of thyroid, neonates < 48hrs of life, patients with prolonged or severe cardiogenic shock, prolonged severe organ perfusion anomalies. In bacteraemic sepsis, PCT is almost always > 2 ng/mL. Thus, PCT is an important marker enabling specific differentiation between a bacterial infection and other causes of inflammatory reactions. Please correlate with patient history, treatment history & results of any other tests performed.

Related tests: Biofire panels, Culture & sensitivity tests.

****End Of Report****



Dr. Devashis Mandal

MBBS, MD (Pathology), Consultant - Lab Services, Reg No: 66456 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740018
UHID	390518	Sample Date	27/06/2025 2:50PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 3:42PM
Bed No/Ward	ICU-10-5F	Report Dates	27/06/2025 5:33PM
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Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

Troponin - I (HS), serum

Troponin I (High Sensitive)

29.6

ng/L

0.0 - 19.0

Sample: Serum
ELFA (Enzyme-Linked Fluorescent Assay) technique.

Cardiac Troponin I in human serum is measured using the ELFA (enzyme-linked fluorescent assay) technique. Troponin I is a regulatory protein of the thin filament of striated muscle, and consists of three subunits I, T & C, which are implicated together in muscle contractions. Troponin I has a cardiac isoform which enables highly specific detection of myocardial injury. This isoform is rapidly released after acute myocardial infarction (AMI) and can be detected in blood between the 4th and 8th hour after the onset of chest pain, with a peak between the 14th and 36th hour; concentrations in blood remain high for 3 to 7 days.

End Of Report

Vandana Raut

Dr. Vandana Raut

MBBS, MD (Pathology), Senior Consultant - Lab Services,
Reg No: 80135 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740018
UHID	390518	Sample Date	27/06/2025 2:50PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 3:43PM
Bed No/Ward	ICU-10-5F	Report Dates	27/06/2025 4:52PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Microbiology

Serology Rapid Screening (HCV, HBSAG & HIV 1&2)

Sample: Serum

HEPATITIS B SURFACE ANTIGEN : NON-REACTIVE

Test technique: RIT(Rapid immunochromatography test)

Note: This is a screening test based on immunochromatography. False positives and false negatives might occur. Kindly perform sensitive and/or confirmatory tests if clinically indicated.

HEPATITIS C ANTIBODY (Anti -HCV) : NON-REACTIVE

Test method: Rapid immunochromatography (RIT)

Note: This is a screening test based on immunochromatography. False positives and false negatives might occur. Kindly do more sensitive and/or confirmatory tests if clinically indicated.

HIV 1 & 2 ANTIBODIES SCREENING TEST : NON-REACTIVE

Test method: RIT (Rapid immunochromatographic test)

Note: This is a screening test based on immunoconcentration. False positives and false negatives might occur. Kindly do more sensitive and/or confirmatory tests if clinically indicated.

****End Of Report****

Dr. Partha Sarathi Mondal
MD Microbiology, REGD: 59958 [WBMC]

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740166
UHID	390518	Sample Date	27/06/2025 6:31PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 7:07PM
Bed No/Ward	ICU-10-5F	Report Dates	28/06/2025 9:57AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Clinical Biochemistry

Serum Sodium

Sodium (Serum) **122** mmol/L 136.0 - 145.0 ISE Sample: Serum

Serum Potassium

Potassium (Serum) **7.1** mmol/L 3.5 - 5.2 ISE Sample: Serum

End Of Report

A. Lodh
Dr. Mousumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab
Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740166
UHID	390518	Sample Date	27/06/2025 6:31PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 7:07PM
Bed No/Ward	ICU-10-SF	Report Dates	30/06/2025 2:32PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Microbiology

Urine C/S, Bacterial

Urine Culture result:

No growth after 48 hours of aerobic incubation at 37 degree C.

End Of Report

Dr. Sambit Sahoo

MBBS, MD (Microbiology), Consultant - Microbiologist and
ICO, Reg No: 87249 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740166
UHID	390518	Sample Date	27/06/2025 6:31PM
Age/Gender	65 Yrs/Male	Receiving Date	27/06/2025 7:07PM
Bed No/Ward	ICU-10-5F	Report Dates	28/06/2025 10:27AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Clinical Pathology

Urine Routine Examination

Sample: Urine

A. Physical Examination:

Colour:-	Pale Yellow	Pale Yellow
Volume:	30	mL
Appearance:	Clear	

B. Chemical Examination:

pH:-	5.0	4.5 - 8.0	Strip Test
Specific Gravity:	1.010	1.015 - 1.030	Strip Test
Protein:	Negative	NEGATIVE	Strip test / Sulphosalicylic acid
Glucose:	Negative	NEGATIVE	Strip test / Benedicts
Ketone:	Negative	NEGATIVE	Strip test / Rotheras
Urobilinogen	Negative		
Bilirubin:	Negative	NEGATIVE	Strip test / Fouchet's test
Nitrite:	Negative	NEGATIVE	Strip Test
Hb / Erythrocytes	Negative		

C. Microscopic Examination:

Leukocytes:	0-1/HPF	Less than 5/HPF	Light microscopy
Epithelial Cells :	Occasional/HPF	Few /HPF	Light microscopy
RBC:	Nil/HPF	0-2 /HPF	Light microscopy
Crystals [per hpf]	Nil	NIL	
Casts:	Nil	NIL	Light microscopy

End Of Report

Dr. Sankha Subhra Sinha
MBBS, MD (Pathology), Consultant - Lab Services,
Reg No: 68345 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740276
UHID	390518	Sample Date	28/06/2025 4:44AM
Age/Gender	65 Yrs/Male	Receiving Date	28/06/2025 5:19AM
Bed No/Ward	ICU-10-5F	Report Dates	28/06/2025 11:41AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-I healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Haematology

CBC (Complete Blood Count)

Sample: EDTA Whole Blood

Hemoglobin	9.9	g/dL	13.0 - 16.0	Cyanide-free sodium lauryl sulphate (SLS)
Red Blood Cell [RBC] Count	4.05	10 ⁶ /μL	4.50 - 5.50	Electrical resistant impedance
Packed cell volume [PCV]/Haematocrit [HCT]	33.7	%	36.0 - 46.0	Manual/ Calculated
MCV	83.2	fL	83.0 - 101.0	Manual/ Calculated
MCH	24.4	pg	27.0 - 32.0	Manual/ Calculated
MCHC	29.4	g/dL	31.5 - 34.5	Manual/ Calculated
RDW CV	20.3	%	11.5 - 15.0	Calculated
Platelet count	155	10 ³ /μL	150 - 410	Electrical resistant impedance
	Rechecked			
Total Leucocyte Count [TLC]	12.56	10 ³ /μL	4.00 - 10.00	Fluorescence flow cytometry

Differential Leucocyte Count [DLC]

Neutrophils	88.6	%	40 - 80	Electrical impedance & Diff channel scattergram
Lymphocytes	9.2	%	20 - 40	Electrical impedance & Diff channel scattergram
Monocytes	1.8	%	2.0 - 10.0	Electrical impedance & Diff channel scattergram
Eosinophils	0.2	%	1 - 6	Electrical impedance & Diff channel scattergram
Basophils	0.2	%	0 - 1	Electrical impedance & Diff channel scattergram
Neutrophil-to-Lymphocyte ratio [NLR]	9.63			Calculated

Remarks: Test repeated on fresh sample.

End Of Report

Vandana Raut

Dr. Vandana Raut

MBBS, MD (Pathology), Senior Consultant - Lab Services,
Reg No: 80135 (WBMC)

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501/1832

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740276
UHID	390518	Sample Date	28/06/2025 4:44AM
Age/Gender	65 Yrs/Male	Receiving Date	28/06/2025 5:19AM
Bed No/Ward	ICU-10-5F	Report Dates	28/06/2025 9:58AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

Serum Urea

Urea **86.7** mg/dL 18 - 55 Sample: Serum
Urease, UV

Serum Creatinine

Serum Creatinine **1.22** mg/dL 0.67 - 1.20 Sample: Serum
Jaffé, Kinetic
For Confirmation, please check KFT [Test code 32516], eGFR [Test code],UACR [Test code 5323]

Electrolytes

Sodium (Serum) **129** mmol/L 136.0 - 145.0 Sample: Serum
ISE
Potassium (Serum) **6.5** mmol/L 3.5 - 5.2 ISE
Chloride (Serum) **108** mmol/L 98.0 - 106.0 ISE

Serum Calcium (Total)

Calcium **8.73** mg/dL 8.80 - 10.20 Sample: Serum
BAPTA

phosphorus (Serum)

Phosphorus (Serum) **4.42** mg/dL 2.60 - 4.50 Sample: Serum
Phosphomolybdate

Uric Acid, serum

Uric Acid **8.58** mg/dL 4 - 7 Sample: Serum
Uricase

Uric acid is the end product of protein metabolism. High levels are seen with kidney disease, alcohol intake, gout, inherited metabolic disorders of purine metabolism, excessive purine dietary intake & increased cell turnover. Not all patients with hyperuricemia have gout. Drugs contributing to hyperuricemia are the primary antiuricosurics, diuretics, salicylates, pyrazinamide, ethambutol, nicotinic acid, cyclosporin, cytotoxic agents, etc.

M. Lodh
Dr. Moushumi Lodh

MBBS, MD (Biochemistry), MHA, Executive Director- Lab Services, Healthworld Hospitals, Reg No: 71993 (WBMCI)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740276
UHID	390518	Sample Date	28/06/2025 4:44AM
Age/Gender	65 Yrs/Male	Receiving Date	
Bed No/Ward	ICU-10-5F	Report Dates	
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740816
UHID	390518	Sample Date	28/06/2025 6:12PM
Age/Gender	65 Yrs/Male	Receiving Date	28/06/2025 6:40PM
Bed No/Ward	ICU-10-5F	Report Dates	29/06/2025 11:30AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Clinical Biochemistry

Serum Sodium					Sample: Serum
Sodium (Serum)	127.3	mmol/L	136.0 - 145.0	ISE	
Serum Potassium					Sample: Serum
Potassium (Serum)	6.05	mmol/L	3.5 - 5.2	ISE	

End Of Report

Vandana Raut

Dr. Vandana Raut

MBBS, MD (Pathology), Senior Consultant - Lab Services,
Reg No: 80135 (WBMC)

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NABL-1032

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740903
UHID	390518	Sample Date	29/06/2025 4:01AM
Age/Gender	65 Yrs/Male	Receiving Date	29/06/2025 5:46AM
Bed No/Ward	ICU-10-5F	Report Dates	29/06/2025 12:33PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur- 16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Haematology

CBC (Complete Blood Count)

Sample: EDTA Whole Blood

Hemoglobin	9.4	g/dL	13.0 - 16.0	Cyanide-free sodium lauryl sulphate (SLS)
Red Blood Cell [RBC] Count	3.94	$10^6/\mu\text{L}$	4.50 - 5.50	Electrical resistant impedance
Packed cell volume [PCV]/Haematocrit [HCT]	32.6	%	36.0 - 46.0	Manual/ Calculated
MCV	82.7	fL	83.0 - 101.0	Manual/ Calculated
MCH	23.9	pg	27.0 - 32.0	Manual/ Calculated
MCHC	28.8	g/dL	31.5 - 34.5	Manual/ Calculated
RDW CV	19.9	%	11.5 - 15.0	Calculated
Platelet count	147	$10^3/\mu\text{L}$	150 - 410	Electrical resistant impedance
	Rechecked			
Total Leucocyte Count [TLC]	8.83	$10^3/\mu\text{L}$	4.00 - 10.00	Fluorescence flow cytometry

Differential Leucocyte Count [DLC]

Neutrophils	84.9	%	40 - 80	Electrical impedance & Diff channel scattergram
Lymphocytes	13	%	20 - 40	Electrical impedance & Diff channel scattergram
Monocytes	1.6	%	2.0 - 10.0	Electrical impedance & Diff channel scattergram
Eosinophils	0.3	%	1 - 6	Electrical impedance & Diff channel scattergram
Basophils	0.2	%	0 - 1	Electrical impedance & Diff channel scattergram
Neutrophil-to-Lymphocyte ratio [NLR]	6.531			Calculated

End Of Report

Dr. Vandana Raut

MBBS, MD (Pathology), Senior Consultant - Lab Services,
Reg No: 80135 (WBMCI)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740903
UHID	390518	Sample Date	29/06/2025 4:01AM
Age/Gender	65 Yrs/Male	Receiving Date	29/06/2025 5:46AM
Bed No/Ward	ICU-10-5F	Report Dates	29/06/2025 12:00PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Clinical Biochemistry				
Serum Urea				
Urea	65	mg/dL	18 - 55	Urease, UV Sample: Serum
Serum Creatinine				
Serum Creatinine	1.04	mg/dL	0.67 - 1.20	Jaffe, Kinetic Sample: Serum
Please correlate clinically				
Electrolytes				
Sodium (Serum)	131	mmol/L	136.0 - 145.0	ISE Sample: Serum
Potassium (Serum)	6.7	mmol/L	3.5 - 5.2	TSE
Chloride (Serum)	110	mmol/L	98.0 - 106.0	TSE
Serum Calcium (Total)				
Calcium	8.63	mg/dL	8.80 - 10.20	BAPTA Sample: Serum
phosphorus (Serum)				
Phosphorus (Serum)	4.53	mg/dL	2.60 - 4.50	Phosphomolybdate Sample: Serum
Uric Acid, serum				
Uric Acid	7.88	mg/dL	4 - 7	Uricase Sample: Serum

Uric acid is the end product of protein metabolism. High levels are seen with kidney disease, alcohol intake, gout, inherited metabolic disorders of purine metabolism, excessive purine dietary intake & increased cell turnover. Not all patients with hyperuricemia have gout. Drugs contributing to hyperuricemia are the primary antiuricosurics, diuretics, salicylates, pyrazinamide, ethambutol, nicotinic acid, cyclosporin, cytotoxic agents, etc.

End Of Report

Vandana Raut

Dr. Vandana Raut

MBBS, MD (Pathology), Senior Consultant - Lab Services,
Reg No: 80135 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1740903
UHID	390518	Sample Date	29/06/2025 4:01AM
Age/Gender	65 Yrs/Male	Receiving Date	
Bed No/Ward	ICU-10-5F	Report Dates	
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741149
UHID	390518	Sample Date	29/06/2025 8:03PM
Age/Gender	65 Yrs/Male	Receiving Date	29/06/2025 8:36PM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 9:04AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals, Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Clinical Biochemistry

Serum Sodium

Sodium (Serum)

133

mmol/L

136.0 - 145.0

ISE

Sample: Serum

Serum Potassium

Potassium (Serum)

6.2

mmol/L

3.5 - 5.2

ISE

Sample: Serum

End Of Report

Dr. Moushumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab
Services, Healthworld Hospitals, Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 5:37AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 12:41PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Haematology

CBC (Complete Blood Count)

Sample: EDTA Whole Blood

Hemoglobin	9.5	g/dL	13.0 - 16.0	Cyanide-free sodium lauryl sulphate (SLS)
Red Blood Cell [RBC] Count	3.90	$10^6/\mu\text{L}$	4.50 - 5.50	Electrical resistant impedance
Packed cell volume [PCV]/Haematocrit [HCT]	32.6	%	36.0 - 46.0	Manual/ Calculated
MCV	83.6	fL	83.0 - 101.0	Manual/ Calculated
MCH	24.4	pg	27.0 - 32.0	Manual/ Calculated
MCHC	29.1	g/dL	31.5 - 34.5	Manual/ Calculated
RDW CV	19.9	%	11.5 - 15.0	Calculated
Platelet count	131	$10^3/\mu\text{L}$	150 - 410	Electrical resistant impedance
	Rechecked			
Total Leucocyte Count [TLC]	7.82	$10^3/\mu\text{L}$	4.00 - 10.00	Fluorescence flow cytometry

Differential Leucocyte Count [DLC]

Neutrophils	82.6	%	40 - 80	Electrical impedance & Diff channel scattergram
Lymphocytes	15	%	20 - 40	Electrical impedance & Diff channel scattergram
Monocytes	1.8	%	2.0 - 10.0	Electrical impedance & Diff channel scattergram
Eosinophils	0.3	%	1 - 6	Electrical impedance & Diff channel scattergram
Basophils	0.3	%	0 - 1	Electrical impedance & Diff channel scattergram
Neutrophil-to-Lymphocyte ratio [NLR]	5.507			Calculated

End Of Report

Dr. Sankha Subhra Sinha

MBBS, MD (Pathology), Consultant - Lab Services,
Reg No: 68345 (WBMC)

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For Blood Collection from Home, Contact: 8170052815 (Durgapur), 8170052700 (Asansol), 8170052680 (Dhanbad)



DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 5:37AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 12:41PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-I healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Haematology

Reticulocyte Count, plasma

Reticulocyte count	3.16	%	0.500 - 2.500	Sample: EDTA Whole Blood Supra Vital stain with Light Microscopy
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End Of Report

Dr. Sankha Subhra Sinha
MBBS, MD (Pathology), Consultant - Lab Services,
Reg No: 68345 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 5:37AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 12:41PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Haematology

Peripheral Blood Smear (PBS)

Sample: EDTA Whole Blood

RBC Morphology	Microcytic hypochromic erythrocytes with moderate anisopoikilocytosis. Polychromasia noted.	Leishman stained smear, microscopy
WBC Morphology	Both granulocytes and lymphocytes have morphology and count within normal limits.	Leishman stained smear, microscopy
Platelet	Adequate on smear.	Leishman stained smear, microscopy
Comments	Microcytic hypochromic anaemia.	

End Of Report

Vandana Raut

Dr. Vandana Raut

MBBS, MD (Pathology), Senior Consultant - Lab Services,
Reg No: 80135 (WBMC)

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NHL-1432

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 5:37AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 9:10AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Clinical Biochemistry

Serum Urea				Sample: Serum
Urea	54.6	mg/dL	18 - 55	Urease,UV

Serum Creatinine				Sample: Serum
Serum Creatinine	1.13	mg/dL	0.67 - 1.20	Jaffe, Kinetic
For Confirmation, please check KFT [Test code 32516], eGFR [Test code],UACR [Test code 5323]				

Serum LDH (Lactate Dehydrogenase)				Sample: Serum
Lactate Dehydrogenase (LDH).	213.00	U/L	135.00 - 225.00	IFCC Lactate to Pyruvate

Elevated serum levels of LDH have been observed in a variety of disease states. The highest levels are seen in patients with megaloblastic anemia,disseminated carcinoma and shock. Moderate increases occur in muscular disorders, nephrotic syndrome and cirrhosis. Mild increases in LDH activity have been reported in cases of myocardial or pulmonary infarction,leukemia, hemolytic anemia and non-viral hepatitis,etc. Serum LDH levels used in conjunction with serial serum lactate values may be reliable markers to differentiate patients with progressive lymphomatous disease from patients with lymphoma with sepsis only.

Serum TSH (Thyroid Stimulating Hormone)				Sample: Serum
Thyroid Stimulating Hormone (TSH)/Thyrotropin	1.23	µIU/mL	0 - 4	

Dr. Moushumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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MC-1002

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 5:37AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 9:10AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

TSH Values :

Premature 28-36 weeks: 0.7-27 uIU/mL;

Cord blood (>37 Weeks) 2.3-13.2 uIU/mL

Pregnancy :1st trimester: 0.3-4.5 uIU/mL, 2nd trimester 0.5-4.6 uIU/mL, 3rd trimester 0.8-5.2 uIU/mL

This test is used in differential diagnosis of hypothyroidism, as an aid in diagnosis of hyperthyroidism, prediction of TRH stimulated TSH response & monitoring patients on thyroid replacement therapy. Drugs like glucocorticoids, dopamine agonists, somatostatin analogs and rexinoids, etc may affect TSH at the level of the hypothalamus or pituitary. In case of high or low TSH, please check FT3, FT4, Thyroid antibodies & correlate with clinical findings, medication history, radiological tests & other endocrinological test reports. Repeat if necessary.

Serum Sodium

Sodium (Serum) **134** mmol/L 136.0 - 145.0 Sample: Serum

Serum Potassium

Potassium (Serum) **5.2** mmol/L 3.5 - 5.2 Sample: Serum

Iron studies (Iron, TIBC, Transferrin Sat., Ferritin)

Iron	19.0	ug/dl	59.0 - 158.0	Sample: Serum
Un-Saturated Iron Binding Capacity	214.10	ug/dl	125.00 - 345.00	
Total Iron Binding Capacity [TIBC]	233.1	ug/dl	224 - 428	
Transferrin Saturation	8.151	%	20 - 50	
Ferritin	50.43	ng/mL	30.00 - 400.00	EDTA

****End Of Report****

Dr. Moushumi Lodh

MBBS, MD (Biochemistry), MHA, Executive Director- Lab Services, Healthworld Hospitals, Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 5:37AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 9:10AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

Folic Acid (Serum)	20.00	ng/mL	5.40 - 18.00	EQ1A
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Sample: Serum

Decreased folate levels are seen in chronic alcoholism, inadequate dietary intake, malnutrition, megaloblastic anemia. Interference may occur from biotin supplements, methotrexate therapy, etc.

Vitamin B12	2000	pg/mL	279.0 - 996.0	EQ1A
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Sample: Serum

****End Of Report****

A. Lodh
Dr. Moushumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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Website : www.healthworldhospitals.com, Email : info@hwhos.com, CIN : U85100WB2014PTC201736 PAN : AAHCP6873G
For any query, Pl. Call : 8170055555 / 8170066660 / 0343-2547755, For Ambulance, Call : 8170052660
For Blood Collection from Home, Contact : 8170052815 (Durgapur), 8170052700 (Asansol), 8170052680 (Dhanbad)

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1741232
UHID	390518	Sample Date	30/06/2025 4:51AM
Age/Gender	65 Yrs/Male	Receiving Date	30/06/2025 7:20AM
Bed No/Ward	ICU-10-5F	Report Dates	30/06/2025 10:00AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Transfusion Medicine Investigation

Coombs Test, Direct Antiglobulin Test (DAT), GEL (CAT)

Direct Coomb's Test result **NEGATIVE**

Note:

Direct Coomb's Test may be positive in the following conditions:

1. Autoimmune hemolytic anemia
2. Hemolytic disease of newborn
3. hemolytic transfusion reaction
4. Drug induced hemolysis [e.g., Methyl Dopa, 1-Dopa, Penicillin, etc.]

****End Of Report****

Dr. Devashis Mandal

MBBS, MD (Pathology), Consultant - Lab Services, Reg No: 66456 (WBMCI)

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Patient Name: ALOKE KUMAR MUKHERJEE

Referred By

Lab ID : 1741232

Sample date : 30/06/2025

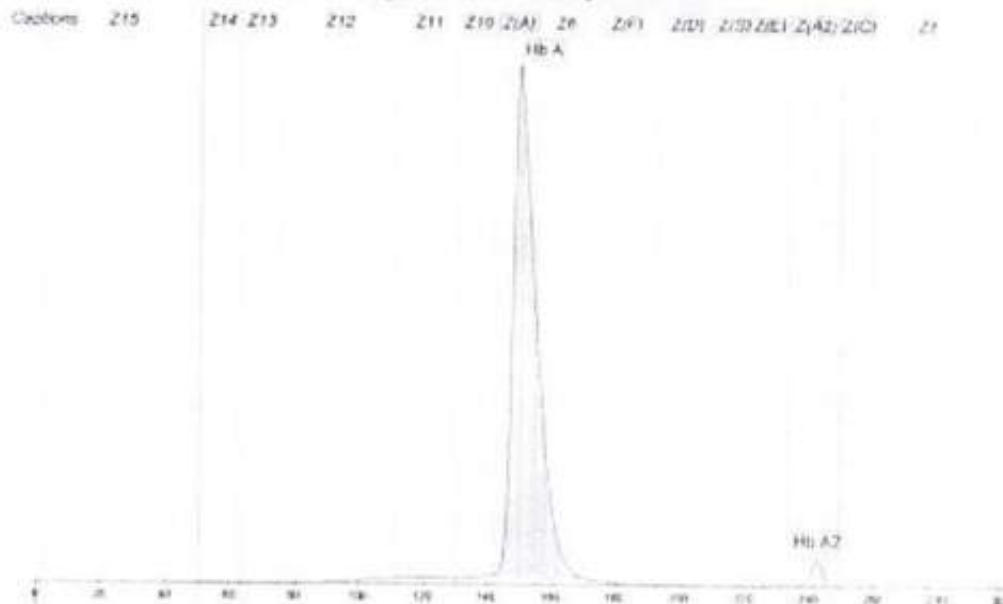
Age : 65

Sex: M

Reporting Date : 30/06/2025

Dept.: Clinical Biochemistry

Haemoglobin Electrophoresis



Name	%	Normal Values %
Hb A	97.5	96.8 - 97.8
Hb A2	2.5	2.2 - 3.2

Comments

No abnormal hemoglobin variant detected. Test may be repeated after correction of iron, vitamin B12, folate deficiency, if any.

Handwritten signature: M. L. 30/6/25

Dr. Mousumi Lodh
MBBS, MD Biochemistry
Lab Services
Healthworld Hospital

Test Performed on "Minicap Flex piercing, fully automated capillary electrophoresis system from Sebia, France

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141-2852

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1742250
UHID	390518	Sample Date	01/07/2025 5:27AM
Age/Gender	65 Yrs/Male	Receiving Date	01/07/2025 6:15AM
Bed No/Ward	General Ward Male - 3 6F	Report Dates	01/07/2025 11:45AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

Glycosylated Haemoglobin (HbA1c) blood

Glycosylated Haemoglobin [HbA1c] in % 4.30

Sample: EDTA Whole Blood

HbA1c by HPLC D16

Non diabetic adults
<5.7%
Pre diabetic: 5.7-
6.4%
Diabetes mellitus:
≥ 6.5%

Unit Conversion formula: NGSP in % = (0.0915 X IFCC in mmol/mol) + 2.152

Test Method: HPLC by Biorad D10.

HbA1c test is useful in diagnosis of diabetes mellitus. It is better at monitoring diabetic patients' compliance & long term blood glucose level control. It reflects the mean glucose concentration over the previous 6-12 weeks & predicts development & progression of diabetic microvascular complications. Values may not be comparable with different methodologies & even different labs using same methodology. Increased if HbF is more than normal, increased in patients with CRF, IDA, splenectomy, increased triglycerides, alcohol, lead & opiate toxicity, salicylate treatment. Chronic large dose use of aspirin can acetylate hemoglobin, & chronic opiate use can interfere with assays to lead to falsely elevated HbA1c. HbA1c is decreased in conditions like hemolytic anemia, blood loss, after transfusion, pregnancy, ingestion of large amounts of Vitamin C / E, Hemoglobinopathies, etc. Dapsone, Ribavirin, Antiretroviral agents and Sulphonamides, cause false low HbA1C while Hydroxyurea promotes a shift in the hemoglobin pattern from HbA to HbF, with apparent decrease in HbA1c levels.

In the absence of unequivocal hyperglycemia, diagnosis requires two abnormal test results from the same sample or in two separate test samples.

If HbA1c is reported as 'not processed', please request for test for abnormal hemoglobin variants. Test Code 32554.

Lipid Profile Basic [PANEL] (T.Chol, Trig, HDL, LDL, VLDL) (Fasting)

Sample: Serum

Total Cholesterol 40 mg/dL 0 - 200

CHO-P00

Dr. Moushumi Lodh

MBBS, MD (Biochemistry), MHA, Executive Director- Lab Services, Healthworld Hospitals, Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE		Lab No	1742250
UHID	390518		Sample Date	01/07/2025 5:27AM
Age/Gender	65 Yrs/Male		Receiving Date	01/07/2025 6:15AM
Bed No/Ward	General Ward Male - 3 6F		Report Dates	01/07/2025 11:45AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay			
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16		Processed At	Lab-Healthworld Hospitals,Durgapur-16
Triglyceride	42	mg/dL	0 - 150	Enzymatic colorimetry
HDL Cholesterol	24.6	mg/dL	40 - 60	Homogenous, Enzymatic
LDL Cholesterol	11.5	mg/dL	0 - 100	Homogenous, Enzymatic
VLDL Cholesterol	8.4	mg/dL	10 - 38	Calculated
HDL / LDL Cholesterol Ratio	2.14	Ratio	0.30 - 0.40	Calculated
Total Cholesterol / HDL Ratio	1.63	Ratio	3.30 - 4.40	Calculated
Serum Sodium				Sample: Serum
Sodium (Serum)	131	mmol/L	136.0 - 145.0	ISE
Serum Potassium				Sample: Serum
Potassium (Serum)	6.1	mmol/L	3.5 - 5.2	ISE

End Of Report

Dr. Mousumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1742250
UHID	390518	Sample Date	01/07/2025 5:27AM
Age/Gender	65 Yrs/Male	Receiving Date	01/07/2025 6:15AM
Bed No/Ward	General Ward Male - 3 6F	Report Dates	01/07/2025 11:45AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-I healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

Albumin/Creatinine Ratio, Urine Spot (ACR, UACR)

Sample: Urine

Urine albumin	2.31	mg/dL	0 - 2
Urine Creatinine in mg/dL	55.55	mg/dL	20.00 - 320.00
Urine albumin Creatinine Ratio [ACR, UACR],	41.584	ug/mg	Normal 0-30 ug/mg Moderately increased albuminuria 30-300 ug/mg Severely increased albuminuria >300 ug/mg

For Confirmation, please check KFT [Test code 32516], eGFR [Test code],UACR [Test code 5323]

****End Of Report****

Dr. Mousumi Lodh
Dr. Mousumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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NABL-1832

DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1742326
UHID	390518	Sample Date	01/07/2025 8:40AM
Age/Gender	65 Yrs/Male	Receiving Date	01/07/2025 8:57AM
Bed No/Ward	General Ward Male - 3 6F	Report Dates	01/07/2025 11:45AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-I Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry**Serum Calcium (Total)**

Calcium	7.78	mg/dL	8.80 - 10.20	Sample: Serum BAPTA
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End Of Report

Dr. Moushumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab
Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1743040
UHID	390518	Sample Date	01/07/2025 7:10PM
Age/Gender	65 Yrs/Male	Receiving Date	01/07/2025 8:29PM
Bed No/Ward	General Ward Male - 3 6F	Report Dates	02/07/2025 9:36AM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
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Clinical Biochemistry

Serum Potassium

Potassium (Serum)

5.3

mmol/L

3.5 - 5.2

ISE

Sample: Serum

End Of Report

Dr. Mousumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1743184
UHID	390518	Sample Date	02/07/2025 5:24AM
Age/Gender	65 Yrs/Male	Receiving Date	02/07/2025 12:41PM
Bed No/Ward	General Ward Male - 3-6F	Report Dates	03/07/2025 6:36PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16

Test Name	Result	Unit	Bio. Ref. Range	Method
-----------	--------	------	-----------------	--------

Clinical Biochemistry

Serum Sodium

Sodium (Serum)	135.1	mmol/L	136.0 - 145.0	ISE	Sample: Serum
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Serum Potassium

Potassium (Serum)	5.00	mmol/L	3.5 - 5.2	ISE	Sample: Serum
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End Of Report

Dr. Moushumi Lodh

MBBS,MD (Biochemistry), MHA, Executive Director- Lab
Services,Healthworld Hospitals,Reg No: 71993 (WBMC)

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DEPARTMENT OF LABORATORY SERVICES

Patient Name	MR. ALOKE KUMAR MUKHERJEE	Lab No	1743184
UHID	390518	Sample Date	02/07/2025 5:24AM
Age/Gender	65 Yrs/Male	Receiving Date	02/07/2025 7:05AM
Bed No/Ward	General Ward Male - 3 6F	Report Dates	03/07/2025 6:36PM
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Report Status	Final
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay		
Collected At	Healthworld Hospitals, City Centre, Durgapur - 16	Processed At	Lab-Healthworld Hospitals,Durgapur-16
Test Name	Result	Unit	Bio. Ref. Range Method

Clinical Biochemistry

ACTH (Adrenocorticotrophic Hormone) (TEST OUTSOURCED) (M)

ACTH **22.9** pg/mL

ACTH Remarks

Plasma levels of ACTH exhibit a significant diurnal variation with high levels in the morning and low levels in the evening. Conditions like pregnancy, menstruation and stress increase secretion of ACTH. Elevated levels of ACTH are seen in Addison's disease (primary adrenal insufficiency), congenital Adrenal Hyperplasia (CAH), ectopic ACTH producing tumors, Nelson's Syndrome, Cushing's disease. Low levels of ACTH are seen in secondary adrenal insufficiency, adrenal carcinoma, adenoma, hypopituitarism. Patients on biotin supplement may have interference in some immunoassays. With individuals taking high dose biotin supplements (> 5 mg per day) at least an 8 hour wait time before a blood draw is recommended.

Associated tests Cortisol AM (test code 1089), Cortisol PM (Test code 29791), Cortisol stimulation by ACTH (test code 25995)

****End Of Report****

Dr. Mousumi Lodh
Dr. Mousumi Lodh

**MBBS,MD (Biochemistry), MHA, Executive Director- Lab
Services,Healthworld Hospitals,Reg No: 71993 (WBMC)**

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Mr. ALOKE KUMAR MUKHERJEE

PID NO: P24125548942054

Age: 65 Year(s) Sex: Male



Reference: DR.P R NAYAK

Sample Collected At:
Healthworld Hospitals (A Unit Of
Parasmani Medica
C-49 Commercial Area, Nr Gandhi More
Beside Pathik, Opp Esic Sub-regional
Processing Location:- Metropolis
Healthcare Ltd,Unit No409-416,4th
Floor,Commercial Building-1,Kohinoor
Mall,Mumbai-70

VID: 250282505853300

Registered On:

03/07/2025 04:49 PM

Collected On:

03/07/2025 10:11AM

Reported On:

05/07/2025 05:44 PM

Investigation

Observed Value

Unit

Biological Reference Interval

Aldosterone / PRA Ratio

PRA (Plasma Renin Activity)

(Plasma,Chemiluminescence Immunoassay
(CLIA))

2.35

ng/ml/hr

Lying Position: 0.15-2.33

Standing Position: 0.10-6.56

Interpretation-

Clinical Condition	Plasma renin activity	Aldosterone
Primary hyperaldosteronism	Decreased	Increased
Malignant hypertension, Renovascular hypertension	Increased	Increased
Cushing syndrome, licorice ingestion, Exogenous mineral corticoids, Liddle's syndrome, congenital adrenal hyperplasia	Decreased	Decreased
Adrenal insufficiency (Addison's disease), Late-onset congenital adrenal hyperplasia, high sodium diet	Increased	Decreased

Limitations:

- It is recommended to stop using B blockers, vasodilators, diuretics, steroids, licorice, and other materials that interfere with renin for two weeks before measuring PRA.
- PRA levels may be affected by sodium intake, so patients should reduce salt intake for 3 days before the PRA test.

Clinical Utility:

- Used in the diagnosis and treatment of hypertension like primary aldosteronism, and unilateral renal artery stenosis.

Associated Tests:

- R0013- Renin Direct Plasma, A0616- Aldosterone/Direct Renin Ratio,

Reference:

- Kit Insert
- Wallach's Interpretation of Diagnostic Tests -10th Edition

Aldosterone / PRA Ratio

(Serum)

4

< 20

Ratio of >20 is suggestive of hyperaldosteronism along with aldosterone >15ng/dl

Dr. AMIT TAORI
MD (Pathology)
Consultant Pathologist
Reg No.2017115350



Mr. ALOKE KUMAR MUKHERJEE

PID NO: P24125548942054
Age: 65 Year(s) Sex: Male



Reference: DR.P R NAYAK
Sample Collected At:
Healthworld Hospitals (A Unit Of
Parasmani Medica
C-49 Commercial Area, Nr Gandhi More
Beside Pathik, Opp Esic Sub-regional
Processing Location:- Metropolis
Healthcare Ltd,Unit No409-416,4th
Floor,Commercial Building-1,Kohinoor
Mall,Mumbai-70

VID: 250282505853300
Registered On:
03/07/2025 04:49 PM
Collected On:
03/07/2025 10:11AM
Reported On:
05/07/2025 05:44 PM

Investigation	Observed Value	Unit	Biological Reference Interval
Aldosterone, Serum (Serum,Chemiluminescence Immunoassay (CLIA))	9.41	ng/dL	Upright: 2.52-39.2 Supine: 1.76-23.2

Interpretation :
1. It is a mineralocorticoid secreted by adrenal gland (zona glomerulosa) and regulates electrolytes (sodium and potassium) which control the blood pressure.
2. It is used for diagnosis of primary hyperaldosteronism, differential diagnosis of fluid and electrolyte disorders, assessment of adrenal aldosterone production.
3. It is increased in primary and secondary hyper aldosteronism. Also increased in pregnancy and people on very low sodium diet.
4. Decreased in congenital adrenal hyperplasia, hypoaldosteronism, Addison's disease, very high sodium diet.

LIMITATIONS:
1. Medicines like hormones (such as progesterone and estrogens), corticosteroids, diuretics, spironolactone (Aldactone), eplerenone (Inspra), and beta-blockers, Black licorice in diet can affect the results. kindly correlate accordingly.
3. Please correlate results with physiological variables such as time of collection, patients posture, and sodium intake.

Clinical Condition	Plasma renin activity	Aldosterone
Primary hyperaldosteronism.	Decreased	Increased
Malignant hypertension, Renovascular hypertension.	Increased	Increased
Cushing syndrome, licorice ingestion, Exogenous mineralocorticoids, Liddle's syndrome, congenital adrenal hyperplasia.	Decreased	Decreased
Adrenal insufficiency (Addison's disease), Late onset congenital adrenal hyperplasia, high sodium diet.	Increased	Decreased

-- End of Report --



Tests marked with NABL symbol are accredited by NABL vide Certificate no MC-2139

Dr. AMIT TAORI
MD (Pathology)
Consultant Pathologist
Reg No.2017115350



DEPARTMENT OF RADIOLOGY & IMAGING

UHID	390518	RIS No	1740070
Patient Name	MR. ALOKE KUMAR MUKHERJEE	Age/Gender	65 Yrs/Male
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Bed No/Ward	ICU-10-5F
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Scan Date	
Bill Date	27/06/2025 3:55PM	Report Date	27/06/2025 5:07PM
Company	Medi Assist Insurance TPA Pvt. Ltd	Report Status	Final

X-RAY: CHEST AP

Increased bronchovascular markings seen in both lower zones.

Both costophrenic angles are clear.

Cardiac shadow appears normal.

IMPRESSION

**** Bronchitic changes.**

-----** End Of Report **-----

Dr. Jayanta Kumar Bokshi
MBBS, DMRD (Cal), Reg No:
70430(WBMC)
Consultant - Radiology



DEPARTMENT OF RADIOLOGY & IMAGING

UHID	390518	RIS No	1740401
Patient Name	MR. ALOKE KUMAR MUKHERJEE	Age/Gender	65 Yrs/Male
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Bed No/Ward	ICU-10-5F
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Scan Date	
Bill Date	28/06/2025 9:46AM	Report Date	30/06/2025 6:01PM
Company	Medi Assist Insurance TPA Pvt. Ltd	Report Status	Final

USG - BEDSIDE

USG WHOLE ABDOMEN (SCREENING)

Liver is moderately enlarged in size with coarse echotexture. Intra-hepatic vascular and biliary radicles appear normal. Right lobe of liver measuring 16 cm.

Portal vein is dilated. Portal vein measuring 18 mm at porta hepatis.

Gallbladder is contracted with multiple calculi and sludge is seen, largest measures 7mm.

Common bile duct is normal in course and caliber. No intraluminal focal lesion or calculus seen. CBD is measuring 4 mm in its max luminal diameter.

Pancreas is normal in position, size, shape, outline and echo pattern. No focal mass lesion seen. Pancreatic duct is not dilated. No calcification seen. No peripancreatic collection seen.

Spleen is moderately enlarged in size. Spleen measuring 19 cm in long axis.

Right Kidney is normal in size, shape, position, outline and axis. Parenchymal echopattern is normal. Cortico-medullary differentiation is maintained. No focal solid or cystic lesion seen. No evidence of renal calculi or hydronephrotic change seen. Right kidney measures: 9.8 x 3.8 cm.

Left Kidney is normal in size, shape, position, outline and axis. Parenchymal echopattern is normal. Cortico-medullary differentiation is maintained. No focal solid or cystic lesion seen. No evidence of renal calculi or hydronephrotic change seen. Left Kidney measures: 10 x 4 cm.

Visualized upper part of both the ureter is not dilated.

Urinary bladder is partially filled.

Prostate gland is normal in size (volume 20 cc.), shape and outline. Homogeneous echo texture is seen. No focal lesion is seen. No calcification is seen. The capsule is intact.

Mild free fluid seen in RIF.

* Content of this report is only an opinion not the diagnosis. Please correlate clinically.
* Report shall not be reproduced except in full and with written approval from the Concern Department.
* For amended reports, the contents are final and supersede the recalled report.

Printed at : 03/07/2025 19:13

A Unit of Parashmani Medical Centre Pvt. Ltd.



DEPARTMENT OF RADIOLOGY & IMAGING

UHID	390518	RIS No	1740401
Patient Name	MR. ALOKE KUMAR MUKHERJEE	Age/Gender	65 Yrs/Male
Referred By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Bed No/Ward	ICU-10-5F
Prescribed By	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Scan Date	
Bill Date	28/06/2025 9:46AM	Report Date	30/06/2025 6:01PM
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USG - BEDSIDE

IMPRESSION:

- Mild hepatomegaly with coarse echotexture, likely CLD (Suggested - CECT abdomen/ LFT correlation).
- Cholelithiasis.
- Dilated PV.
- Moderate splenomegaly.
- Minimal ascites.

--** End Of Report **--

Dr. Snigdha Manual

MBBS, MD(Radio. Diag.), Reg No :
81001 (WBMC)

Consultant- Radiology

QRS
QT/QTcBaz
PR
P
RR/PP
P/ORS/T

164 ms
292 / 343 ms
- ms
- ms
722 / 722 ms
-156 / 27 degrees

Undetermined rhythm
Nonspecific intraventricular block
Lateral infarct, age undetermined
Abnormal ECG

IP No : 25/14029 UHID : 390518

Name : Mr. Aloke Kumar Mukherjee

Age/Sex : 65 Y/M

Bed/Ward : ICU10-5 \ ICU-10-5F / Medi Assist

Insurance

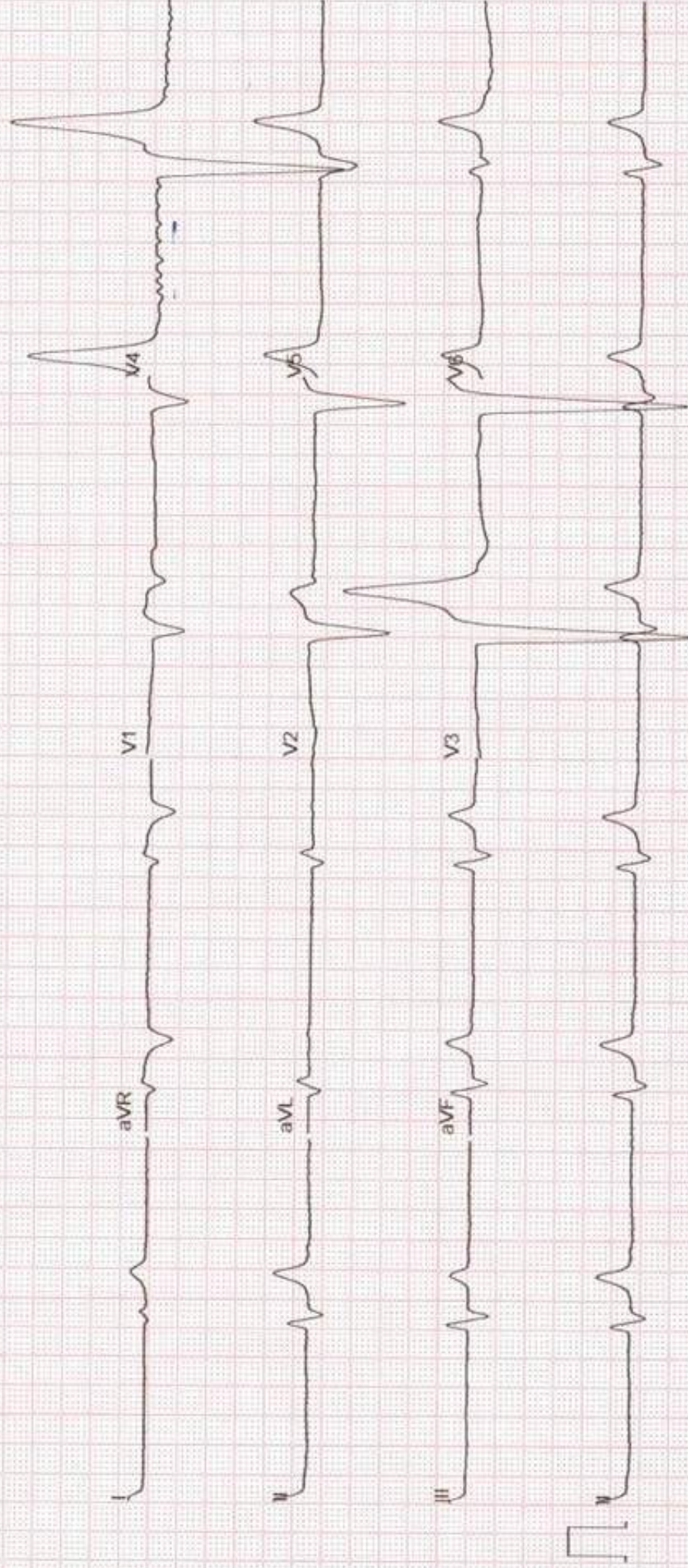
DOA/Category : 27/06/2025--13:51

Doctor : Dr. D K Roy/Dr S Banerjee / Dr P R Nayak /

T Chattopadhyay \ General Medicine

83 bpm

- / - mmHg

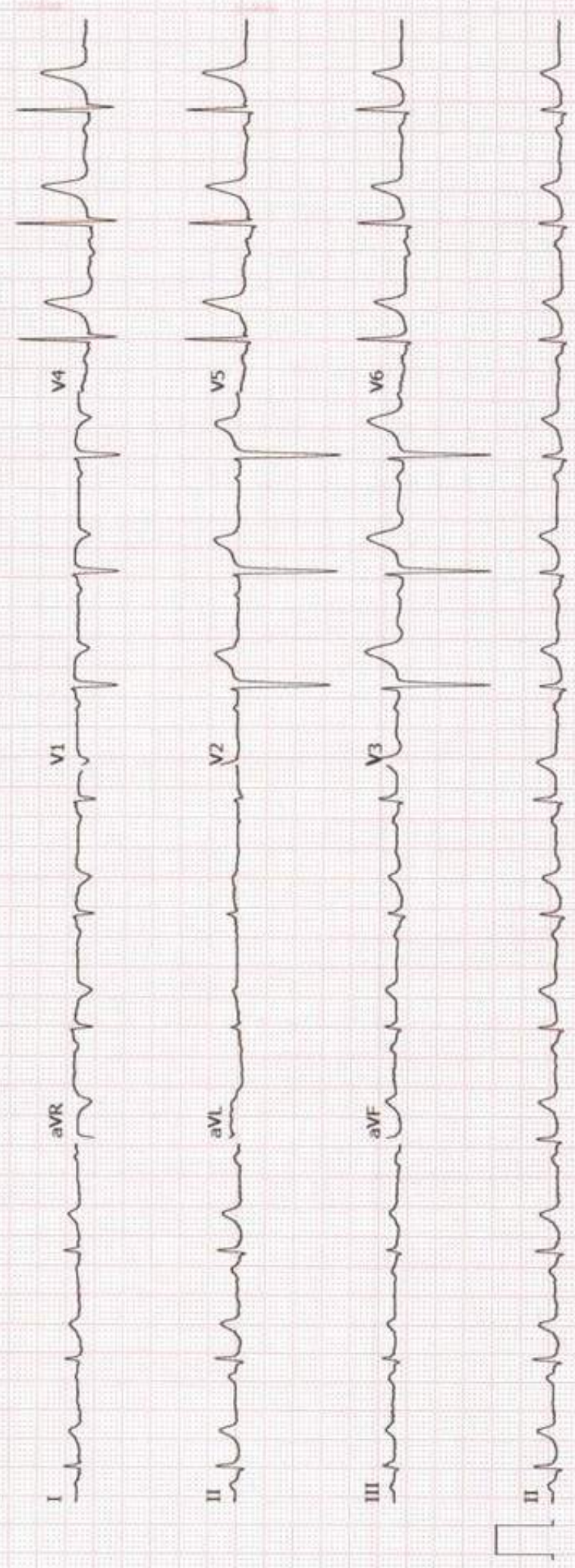


Technician:
Ordering Ph:
Referring Ph:
Attending Ph:

QRS : 78 ms
QT / QTcBaz : 358 / 410 ms
PR : 146 ms
P : 96 ms
RR / PP : 762 / 759 ms
P / QRS / T : 70 / 46 / 53 degrees

Normal sinus rhythm
Low voltage QRS
Borderline ECG

IP No : 25/14029 UHID : 390518
Name : Mr. Alok Kumar Mukherjee
Age/Sex : 65 Y/M
Bed/Ward : ICU-10-5 / ICU-10-SF / Medi Assist
Insuranc
DOA/Category : 27/06/2025--13:51
Doctor : Dr. D K Roy/Dr S Banerjee / Dr P R Nayak /
T Chattopadhyay \General Medicine





DEPARTMENT OF CARDIAC SCIENCES

UHID :	390518	RIS No. :	1739975
Patient Name :	Mr. Aloke Kumar Mukherjee	Age/Gender :	65 Y/M
Prescribed By :	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Bed No/Ward :	ICU-10-5F ICU10-5
Referred By :		Bill Date :	27/06/2025 1:59PM
Report Date :	30/06/2025 11:52AM	Report Status :	Final

ECHOCARDIOGRAM WITH DOPPLER

The patient was in tachycardia with heart rate of 40 bpm during study

SUMMARY

- Concentric LVH.
- Aortic valve sclerosis. Mild TR.
- No RWMA.
- Great arteries normal in size and relation.
- Interatrial and interventricular septum intact.
- Normal LV systolic function.
- Systemic and pulmonary venous drainage normal.
- No PAH.
- Pericardium normal.

FINAL IMPRESSION:

- Concentric LVH.
- No RWMA.
- Mild TR. No PAH.
- Normal LV systolic function.
- No PAH.

Dr. SUBHAJIT MUKHERJEE
MD. (Physician) PGDCC
Associate Consultant
Non Invasive Cardiology
and Nuclear Medicine (MCI)

Dr. Subhajit Mukherjee
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MD (Gen. Med.), DNB (Cardiology)
Dr. Interventional Cardiology
Regn. No. 54676 (WBMC)

Dr. (Major Retd.) Indranil Dev
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Reg No: 73445 (WBMC)



DEPARTMENT OF CARDIAC SCIENCES

UHID :	390518	RIS No. :	1739975
Patient Name :	Mr. Alope Kumar Mukherjee	Age/Gender :	65 Y/M
Prescribed By :	Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr T Chattopadhyay	Bed No/Ward :	ICU-10-5F ICU10-5
Referred By :		Bill Date :	27/06/2025 1:59PM
Report Date :	30/06/2025 11:52AM	Report Status :	Final

ECHOCARDIOGRAM WITH DOPPLER

M-mode Measurements Values :

Aorta	24 mm	LV ed	46 mm
LA	36 mm	LV es	30 mm
RV	22 mm	IVS ed	13 mm
FS	27%	PW (LV)	12 mm
LVEF	55%	IVC	12/6 mm

VALVES:-

MITRAL VALVE

Morphology : AML Normal ;
PML Normal.
No vegetation seen.

Subvalvular deformity (absent) :

Doppler : Normal.

Mitral stenosis : Absent.

Mitral Regurgitation : Absent.

TRICUSPID VALVE

Morphology. No vegetation seen.

Doppler : Abnormal.

Tricuspid Stenosis : Absent.

Tricuspid regurgitation : Mild.

TV Peak Gradient = 22 mm of Hg

PULMONARY VALVE

Morphology : Normal. No vegetation seen.

Doppler : Normal.

Pulmonary stenosis : Absent.

Pulmonary regurgitation : Absent.

PASP = 27 mm of Hg.

AORTIC VALVE

Morphology : Thickened/ Calcified. No vegetation seen.

No of cusps : 3

Doppler : Normal.

Aortic stenosis : Absent.

Aortic regurgitation : Absent.

AV Peak Gradient = 6.2 mm of Hg.

CHAMBERS

Left Ventricle : Concentric LVH.

Left Atrium : Normal ; No clots seen.

Right Atrium : Normal.

Right Ventricle : Normal.

OTHERS

GREAT ARTERIES : Normal in size and relation.

PERICARDIUM : Normal.

ABG REPORT



Patient Details

24/06/25

IP No : 25/14029 UHID : 390518
Name : Mr. Alok Kumar Mukherjee
Age/Sex : 65 Y/M
Bed/Ward : ICU10-S \ ICU-10-5F / Medi Assist
Insurance :
DOA/Category : 27/06/2025--13:51
Doctor : Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / I
T Chattopadhyay \ General Medicine

Instrumentation Laboratory

PATIENT SAMPLE REPORT

HEALTH WORLD HOSPITAL
DURGAPUR

Status: ACCEPTED
27/06/2025 13:11:31
Sample Type:
Arterial
Sample No.: 413
Patient:
Name:
ALOK KRM
Instrument:
Model: GEM 3500
S/N: 91101730

RA

Measured (37.0C)

pH	7.29
pCO2	17 mmHg
pO2	106 mmHg
Na+	112 mmol/L
K+	7.7 mmol/L
Ca++	1.00 mmol/L
Glu	131 mg/dL
Lac	1.2 mmol/L
Hct	42 %

Derived Parameters

Ca++(7.4)	0.96	mmol/L
HCO3-	8.2	mmol/L
HCO3std	12.5	mmol/L
TCO2	8.7	mmol/L
BEecf	-18.4	mmol/L
BE(B)	-15.9	mmol/L
S02c	97	%
THbc	13.0	g/dL

?A-aD02
?pA02
?pa02/pA02
?RI

?=Review

Name : Mr. Alok Kumar Mukherjee

Age/Sex : 65 Y/M

Bed/Ward : ICU10-S \ ICU-10-5F / Medi Assist

Insurance :

DOA/Category : 27/06/2025--13:51

Doctor : Dr. D K Roy/Dr S Banerjee /Dr P R Nayak / Dr
T Chattopadhyay \ General Medicine

Instrumentation Laboratory

PATIENT SAMPLE REPORT

HEALTH WORLD HOSPITAL
DURGAPUR

Status: ACCEPTED
28/06/2025 06:52:09
Sample Type:
Arterial
Sample No.: 343
Patient:
Name:
ALOK KR MUKHERJE
Sex: U
Instrument:
Model: GEM 3500
S/N: 20114514

RA

Measured (37.0C)

pH	7.35
pCO2	24 mmHg
pO2	111 mmHg
Na+	127 mmol/L
K+	5.5 mmol/L
Ca++	0.88 mmol/L
Glu	84 mg/dL
Lac	0.9 mmol/L
Hct	34 %

Derived Parameters

Ca++(7.4)	0.86	mmol/L
HCO3-	13.2	mmol/L
HCO3std	16.5	mmol/L
TCO2	13.9	mmol/L
BEecf	-12.4	mmol/L
BE(B)	-10.9	mmol/L
S02c	98	%
THbc	10.5	g/dL

?A-aD02
?pA02
?pa02/pA02
?RI

?=Review



ABG REPORT

Patient Details

IP No : 25/14029 UHID : 390518
Name : Mr. Alope Kumar Mukherjee
Age/Sex : 65 Y/M
Bed/Ward : ICU10-5 \ ICU-10-SF / Medi Assist
Insurance :
DOA/Category : 27/06/2025--13:51
Doctor : Dr. D K Roy / Dr S Banerjee / Dr P R Nayak /
T Chattopadhyay \ General Medicine

PATIENT SAMPLE REPORT

HEALTH WORLD HOSPITAL
DURGAPUR

Status: ACCEPTED
29/06/2025 06:29:14

Sample Type:
Arterial

Sample No.: 48

Patient:

ID: 390518

Name:

ALOKE

Sex: M

Instrument:

Model: GEM 3500

S/N: 20114514

Measured (37.0C)

pH	7.36	
pCO2	26	mmHg
pO2	109	mmHg
Na+	129	mmol/L
K+	5.6	mmol/L
Ca++	0.93	mmol/L
Glu	91	mg/dL
Lac	0.9	mmol/L
Hct	35	%

Derived Parameters

Ca++(7.4)	0.91	mmol/L
HCO3-	14.7	mmol/L
HCO3std	17.6	mmol/L
TCO2	15.5	mmol/L
BEecf	-10.7	mmol/L
BE(B)	-9.4	mmol/L
SO2c	98	%
THbc	10.9	g/dL
?A-aO2	-----	
?pA02	-----	
?pa02/pA02	-----	
?RI	-----	

?=Review

